

The Neurophysiology of Cybersickness in Virtual Reality: A Systematic Review of Neural Correlates and Neurostimulation

Gaël Van der Lee, François Cabestaing, Hakim Si-Mohammed

Abstract—Cybersickness remains a barrier to virtual reality (VR) adoption, and subjective questionnaires alone are insufficient for system-level diagnosis of its determinants. This systematic review, conducted following the PRISMA 2020 statement, examines neuromarkers of cybersickness in VR: signals derived from non-invasive recording or modulation of brain activity that index or modulate the user’s state. A search of PubMed, Web of Science, and Scopus up to 27 March 2025 identified 92 eligible studies: 77 using electroencephalography, 4 using functional near-infrared spectroscopy, and 11 using neurostimulation (5 galvanic vestibular stimulation, 3 transcranial direct-current stimulation, 2 transcranial alternating-current stimulation, 1 other). No eligible studies using fMRI, MEG, or PET were retrieved. Across electroencephalography studies, cybersickness is associated with increased low-frequency power (delta, theta) and decreased alpha-band power, with topographies varying by paradigm and hardware. At the corpus level, VR display hardware significantly modulates band-power responses (alpha: $p = 0.004$, large effect; beta: $p = 0.015$), indicating that prior contradictions in the literature are partly attributable to hardware confounds rather than to differences in cybersickness per se. A methodological audit of the 38 classification or prediction studies finds that only one reports leave-one-subject-out validation, nineteen do not disclose a sick-versus-non-sick decision threshold, and thirteen report accuracy without sensitivity, specificity, AUC, or F1. Relative to the most recent prior scoping review [1], which covered 33 EEG-only studies up to September 2021, this review covers a larger and more recent corpus, explicitly includes neurostimulation alongside neuroimaging, and contributes corpus-level meta-analytic findings. We outline reporting recommendations and identify closed-loop modulation and individual susceptibility prediction as directions where the existing corpus already provides actionable starting points.

Index Terms—Human-centered computing, Human computer interaction (HCI), Interaction paradigms, Virtual reality.

I. INTRODUCTION

Virtual Reality (VR) technology has permeated numerous sectors beyond entertainment, becoming integral tools in fields such as healthcare, professional training, and education [2]. Generally, VR is defined as a computer-generated simulation that enables users to perceive and interact with a three-dimensional environment through real-time multisensory feedback. A typical VR system includes a head-mounted display for stereoscopic rendering, motion-tracked controllers or sensors for user input, and a computing unit for environment simulation and rendering. However, there is no unifying definition of VR [3].

A barrier to widespread VR adoption is the onset of cybersickness (CS) [4]. Cybersickness is a form of visually induced

motion sickness (VIMS) [5] characterized by symptoms such as nausea, dizziness, and disorientation that arise during or after immersion in a virtual environment [4]. The relationship between cybersickness, VIMS, simulator sickness, and motion sickness is treated in detail in Section II; the present review uses VIMS as the umbrella term when introducing constructs and uses the term each cited study employs when reporting its findings. Cybersickness affects a substantial portion of the user population, with studies reporting prevalence between 60% and 95% [4], degrades user experience, impairs task performance, and can lead to premature session termination [6].

Assessment of cybersickness has historically relied on subjective post-hoc questionnaires such as the Simulator Sickness Questionnaire [7]. Such instruments are retrospective, prone to recall bias, and do not capture the dynamic nature of the experience [1], [8]. This limitation creates a need for objective measures grounded in the user’s physiology and for interventions that can modulate the user’s state during exposure rather than after the fact.

Two converging lines of research have responded by examining the brain. The first records neural activity — primarily electroencephalography (EEG), and to a lesser extent functional near-infrared spectroscopy (fNIRS) — to identify signals that index the onset, severity, or trajectory of cybersickness. The second applies non-invasive brain stimulation, including galvanic vestibular stimulation (GVS), transcranial direct-current stimulation (tDCS), and transcranial alternating-current stimulation (tACS), to modulate the underlying processes that produce sickness. We use the term *neuromarkers* to refer to signals derived from either non-invasive recording or non-invasive modulation of brain activity in this context. For a VR system designer, neuromarkers carry information that subjective questionnaires cannot: continuous, time-resolved indices of user state that can in principle drive adaptive content, comfort thresholds, and closed-loop mitigation.

Chang et al. [1] synthesized 33 EEG studies published before September 2021 in a scoping review and characterized the EEG analysis pipeline used in cybersickness research. The present systematic review differs in four respects. First, the scope extends beyond EEG to include functional near-infrared spectroscopy and non-invasive neurostimulation, consistent with the integrated frame of neuromarkers. Second, the corpus is larger and more recent: 92 studies retrieved up to 27 March 2025 from three curated databases (PubMed, Web of Science, Scopus), compared with 33 studies retrieved from

Web of Science and Google Scholar up to September 2021. Third, the synthesis includes corpus-level inferential statistics on hardware and methodological confounders, in addition to the pipeline characterization shared with prior work. Fourth, this review situates the empirical findings within the broader context of the field: the theoretical frameworks invoked to explain cybersickness, the terminology used across the cybersickness, VIMS, simulator sickness, and motion sickness literatures, and a critical appraisal of recurring methodological shortcomings. Where the prior scoping review concentrates on findings and pipelines, this review additionally examines the conceptual and methodological scaffolding in which those findings are produced.

The contributions of this review are threefold. First, we provide a systematic synthesis of cybersickness neuromarkers in VR across 92 studies, covering neuroimaging and neurostimulation under a unified frame. Second, we report meta-analytic evidence that VR display hardware significantly modulates the band-power response associated with cybersickness ($p = 0.004$ on alpha, large effect; $p = 0.015$ on beta), recasting prior contradictions in the EEG–cybersickness literature as partly attributable to hardware confounds rather than to differences in cybersickness per se. Third, we conduct a methodological audit of the classification literature, finding that of 38 cybersickness classification or prediction studies, only one reports leave-one-subject-out validation, nineteen do not disclose a sick versus non-sick decision threshold, and thirteen report accuracy without sensitivity, specificity, AUC, or F1; we derive concrete reporting recommendations from these gaps for future VR-EEG and VR-stimulation work.

Contributions are too narrow^[TODO:intro]

The remainder of this paper is organized as follows. Section II reviews the relevant theoretical frameworks for cybersickness and the principal neuroimaging and stimulation modalities. Section III describes the review methodology and search strategy. Section IV presents the corpus, the per-modality synthesis, and the corpus-level meta-analytic findings. Section V discusses methodological challenges and outlines future directions. Section VI concludes.

II. THEORETICAL AND METHODOLOGICAL BACKGROUND

A. Terminology and Etiological Frameworks

Before surveying the theories proposed to explain cybersickness, it is useful to delimit the construct itself and to distinguish it from related forms of sickness and from perceptual phenomena that often co-occur with it.

Motion sickness is the broadest construct, encompassing adverse responses to both physical and visually specified motion [9], [10]. Within it, visually induced motion sickness (VIMS) designates sickness elicited primarily by visual motion, including cases where physical body motion is absent or inconsistent with the visual scene [11], [12]. Cybersickness is most often treated as the VR-specific instance of VIMS; simulator sickness, gaming sickness, and related labels denote overlapping exposure contexts rather than distinct symptom categories [13]–[15]. This review adopts the above hierarchy when introducing constructs, but reports each included study

using the terminology its authors chose, in order to avoid imposing a uniform vocabulary on a literature that does not yet share one.

Symptom measures in this literature must also be distinguished from related perceptual phenomena. The dominant instrument is the Simulator Sickness Questionnaire (SSQ), which yields nausea, oculomotor, and disorientation subscales [7]; it is frequently complemented by the single-item Fast Motion Sickness Scale (FMS) for continuous, in-task ratings [16]. Broader diagnostic criteria for motion sickness and VIMS extend these subscales to gastrointestinal, thermoregulatory, arousal, dizziness or vertigo, headache, and visual or oculomotor domains [12]. Distinct from these symptom measures, vection denotes the visually induced illusion of self-motion: it frequently covaries with sickness and figures in several etiological theories, but is not itself a symptom category [17], [18].

Several etiological frameworks have been proposed to explain cybersickness and the wider VIMS phenomenon; they are not mutually exclusive and often emphasise complementary mechanisms.

The most influential of these is *sensory conflict theory*, which attributes sickness to inconsistency between visual, vestibular, and proprioceptive signals, or between current sensory input and the prior exposure history encoded by the observer [19]–[21]. Two refinements within this family further specify the nature of the conflict: subjective vertical mismatch frames it as a discrepancy between sensed and expected gravitational vertical [22], while rest-frame conflict locates it in the observer’s difficulty identifying a stable spatial reference [23]. This family is particularly pertinent to VR, where HMDs routinely present visual self-motion while vestibular and proprioceptive signals indicate little or no corresponding physical movement.

A second framework, *postural instability theory*, locates the cause of sickness not in sensory mismatch but in a sustained inability to maintain postural control under unfamiliar sensorimotor demands [24], [25]. The theory is most directly testable in VR paradigms involving standing, walking, or large-field optic flow, where body sway and control dynamics can be measured objectively and have been reported to precede or accompany subjective sickness [26]. A related strand of recent VR work returns to the role of vection itself, showing that sickness intensifies when visually induced self-motion violates the user’s expected motion or control state [27], [28]; vection is treated here as a perceptual condition that may contribute to sickness rather than as a separate theory of motion sickness in its own right.

A further set of proposals address narrower mechanisms or different levels of explanation. Vergence-accommodation conflict identifies a mismatch between binocular convergence and the fixed focal distance of stereoscopic displays as a source of oculomotor discomfort [29], while oculomotor theories more broadly relate sickness to specific eye-movement patterns, such as nystagmus or ocular torsion, evoked by the visual stimulus [30]. At an evolutionary level, Treisman’s toxin-defense hypothesis explains why disturbed sensory integration should give rise to nausea and emesis, but does not specify the

proximate sensory conditions that trigger them [31]. Finally, predictive-coding formulations recast sensory mismatch as a persistent prediction error between expected and observed input, offering a unifying computational vocabulary for the preceding theories [32].

B. Neural Recording and Modulation Modalities

Cybersickness has historically been assessed through subjective reports, but a growing body of work uses neural recordings, and more recently non-invasive brain stimulation, to characterise the user's state during VR exposure or to actively modulate it.

Electroencephalography (EEG) is the most widely used recording modality in this literature. Its high temporal resolution supports the study of spectral power, event-related potentials, and connectivity dynamics throughout the development of sickness, and its portability makes it compatible with HMD-based and other interactive VR paradigms [26], [33]. These strengths are offset by limited spatial specificity, particularly for deep vestibular and autonomic structures, and by methodological sensitivities, such as motion artifacts, electrode density, reference choice, and baseline selection, which constrain the interpretation of reported effects.

Functional near-infrared spectroscopy (fNIRS) is an emerging optical technique that measures cortical hemodynamic changes via near-infrared light [34], [35]. It offers an intermediate trade-off between temporal and spatial resolution and tolerates moderate movement, making it well suited to VR paradigms such as HMD-based roller-coaster exposure, VR locomotion, or stereoscopic viewing [36]–[39]. Because it relies on hemodynamic rather than electrical signals, its temporal resolution remains substantially lower than that of EEG, and its optical penetration restricts measurement to superficial cortical regions.

Beyond recording, non-invasive neurostimulation has begun to be used to modulate sickness-relevant processes during VR or visually induced motion exposure. Reported approaches include galvanic vestibular stimulation, transcranial direct-current stimulation, transcranial alternating-current stimulation, and related techniques [40]. By directly perturbing vestibular or cortical activity, these methods provide a causal complement to the correlational evidence offered by EEG and fNIRS.

III. REVIEW METHODOLOGY

A. Search Strategy

A systematic literature search was conducted across three databases: PubMed, Web of Science, and Scopus. A publication-date cutoff of 27 March 2025 was applied to PubMed and Web of Science; no earlier date restriction was imposed, so the search captures the full historical record of the field. The query targeted the intersection of three concept blocks: brain-directed measurement or modulation methods, cybersickness and related sickness constructs, and immersive virtual reality. The full query was:

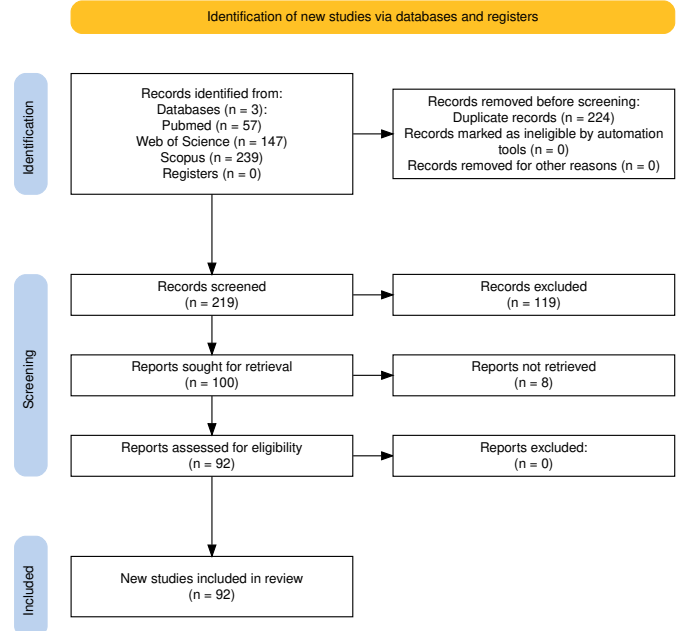


Fig. 1: PRISMA flow diagram for the present review.

```

(("Brain Computer Interface" OR BCI OR EEG OR
electroencephalogram OR ERP OR MEG OR
magnetoencephalography OR fMRI OR ECoG OR
fNIRS OR PET OR "positron emission
tomography" OR neurostimulation OR "brain
stimulation" OR "non-invasive brain
stimulation" OR tACS OR "transcranial
alternating current stimulation" OR tDCS
OR "transcranial direct current
stimulation" OR TMS OR "transcranial
magnetic stimulation" OR "galvanic
vestibular stimulation" OR GVS)
AND (Cybersickness OR "simulator sickness" OR
"visually induced motion sickness" OR VIMS
OR "motion sickness")
AND ("virtual reality" OR VR OR "head-mounted
display" OR HMD OR "immersive environment
"))

```

B. Study Selection and Eligibility Criteria

Records identified by the search were screened in accordance with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) 2020 statement; the full selection process is summarised in the PRISMA flow diagram (Figure 1). The combined search returned 443 records: 57 from PubMed, 147 from Web of Science, and 239 from Scopus. Duplicate removal across the three sources reduced this set to 219 unique records.

Each of the 219 unique records underwent manual title-and-abstract screening against the following eligibility criteria. A record was included if it met all of the following:

- The study employed a neuroimaging modality (EEG, ERP, fNIRS, fMRI, MEG, ECoG, PET) or a non-invasive brain-stimulation modality (tDCS, tACS, TMS, GVS) to measure or modulate brain activity.

- The study addressed cybersickness or a closely related construct (visually induced motion sickness, simulator sickness, or motion sickness in a virtual environment) within a virtual reality or otherwise immersive context.
- The study reported original empirical data from human participants.

A record was excluded if it met any of the following:

- The work was non-empirical (literature reviews, meta-analyses, editorials, letters, or conference abstracts without an accompanying full paper).
- The study did not address cybersickness or a related sickness construct as a primary outcome.
- The full text could not be retrieved.

Title-and-abstract screening excluded 119 records, leaving 100 reports sought for full-text retrieval. Of these, 8 could not be obtained as full text and were excluded at the retrieval stage. The remaining **92 articles** comprise the final corpus analysed in this review.

C. Modalities and Dataset-Reuse Studies

Two aspects of the corpus warrant explicit comment before the synthesis: the modalities that the query covered but that returned no eligible studies, and the inclusion of studies that rely on previously released datasets.

The query included PET, MEG, and fMRI terms alongside the more represented modalities. PET returned no eligible studies; MEG returned a single record, which was excluded at the title-and-abstract stage as not addressing cybersickness or a related sickness construct; fMRI returned no records at all. The absence of these modalities from the final corpus therefore reflects the current literature rather than a scope restriction imposed by the search.

A subset of retrieved studies train machine-learning classifiers on previously released datasets without recording new data. Pure-dataset benchmarking is a related but methodologically distinct literature, since its primary objective is classification performance rather than the identification or modulation of neuromarkers. Six papers in the final corpus reuse one or more publicly released datasets (in two cases combined with newly recorded data); they were retained because they satisfied all eligibility criteria and are flagged where relevant in the synthesis.

D. Data Extraction and Thematic Synthesis

For each of the 92 included articles, a structured data extraction process was performed. This process was guided by a predefined protocol designed to ensure consistency and to facilitate subsequent comparison and synthesis. The following data points were extracted from each paper:

- 1) **Summary of Contribution:** A concise summary of the study's primary objective and its main contribution.
- 2) **Sickness Measurement:** The methods used to measure or label cybersickness, distinguishing between subjective questionnaires (e.g., SSQ, FMS, VRSQ) and objective physiological or behavioural metrics (e.g., galvanic skin response, postural sway).

- 3) **Methodological Pipeline:** A breakdown of the neuro-physiological pipeline, including: (a) the measurement or stimulation modality, (b) the key feature extraction techniques (e.g., power spectral density, event-related potentials, functional connectivity), and (c) the primary analysis or classification methods (e.g., statistical tests, machine-learning models).
- 4) **Theoretical Framework:** The explicitly stated or implicitly assumed theory of cybersickness (e.g., sensory conflict, postural instability) that guided the study's design or interpretation.
- 5) **Quantitative Performance:** Reported performance measures, such as classification accuracy, correlation coefficients, root-mean-square error, or statistical significance values.
- 6) **Empirical Study Details:** For empirical studies, specific details of the experimental protocol, including the VR environment, display hardware, sickness-inducing stimulus, and number of participants.
- 7) **Reported Shortcomings:** Limitations, caveats, or weaknesses identified by the authors or apparent from the study's design.

Following extraction, each paper was assigned to a primary research-objective category to support quantitative synthesis. An initial set of categories was defined *a priori* based on established research paradigms and refined iteratively during extraction. The final categories, which structure the Results section, are: Classification (Binary, Likert, Continuous), Correlates (Brain Regions, Rhythms), Mitigation (A/B Testing, Real-time, Neurostimulation), User Differences, Dataset Provision, Recovery, and studies on the Effects of Cybersickness on other cognitive or performance metrics.

IV. RESULTS

The 92 corpus studies address four main research objectives: classification or prediction of cybersickness from physiological signals (38 studies), identification of neural and physiological correlates (24), development of mitigation strategies (18), and characterisation of individual susceptibility (3). Nine residual studies (cognitive effects, recovery, dataset contributions, other framings) are integrated into the relevant subsections, with full discussion deferred to section V. This section adds two cross-cutting layers on top of these objectives: subsection IV-A describes the corpus as a whole, and subsection IV-F reports meta-analytic observations across the 92 studies.

Activity in the field has accelerated in recent years: 63.0% of the corpus (58 of 92 studies) was published from 2021 onwards, with a corresponding shift toward classification, mitigation, and susceptibility objectives (Figure 2). Per-study sample sizes have also increased over the same period (Mann-Whitney U on Recent versus Earlier studies, Cliff's $\delta = -0.308$, $p = 0.022$, medium effect).

Table I lists all 92 studies with their modality, sample size, virtual reality hardware, stimulus paradigm, cybersickness measure, primary objective, and topic.

TABLE I: All 92 studies included in this systematic review. Modality: EEG, fNIRS, fMRI = neuroimaging; GVS = galvanic vestibular stimulation; tDCS/tACS = transcranial direct/alternating current stimulation; ECG, GSR, EOG, EMG, EGG = peripheral physiology; posturography = balance / centre-of-pressure recording. CS measure abbreviations: SSQ = Simulator Sickness Questionnaire; VRSQ = VR Sickness Questionnaire; FMS = Fast Motion-Sickness Scale; MSSQ = Motion Sickness Susceptibility Questionnaire; cont. = continuous self-report; Likert = single-item Likert; custom = study-specific scale; behav. = behavioural measure only; none = no cybersickness measure reported. Sample size N gives subjects analysed. Topic gives a short noun phrase describing the focus of each paper.

Ref.	Modality	N	VR device	Stimulus	CS measure	Objective	Topic
[41]	EEG	2	HMD	Mixed	custom	Mitigation	EEG-informed scene-simplification mitigation
[42]	EEG+EOG	17	HMD	Driving (passive)	SSQ	Correlates	Alpha connectivity and P3 in susceptibility
[43]	EEG+EOG	33	Screen	Passive viewing	FMS	Correlates	EEG time-frequency correlates of VIMS severity
[44]	EEG	31	HMD	Mixed	SSQ	Correlates	Spectral EEG correlates of VIMS in HMD
[45]	EEG+ECG+GSR+EOG	27	HMD	Active navigation	custom	Classification	Spiking-network EEG+HRV cybersickness prediction
[40]	tACS+GSR	37	HMD	Passive viewing	MSSQ	Mitigation	10 Hz tACS over vestibular cortex
[46]	EEG	68	HMD	Mixed	SSQ	Other	Sex differences in EEG neurofeedback sessions
[47]	EEG+tDCS+ECG+EOG	41	HMD	Passive viewing	SSQ	Mitigation	Placebo tDCS in EEG neurofeedback for cybersickness
[48]	EEG+ECG	39	HMD	Active navigation	SSQ	Cognition	VR-parameter manipulation during EEG neurofeedback
[49]	EEG	15	HMD	Passive viewing	SSQ	Classification	Resting-state EEG entropy classification
[50]	EEG	12	Screen	Passive viewing	SSQ	Mitigation	Rest-frame mitigation with EEG correlates
[51]	EEG+ECG+EOG	20	HMD	Passive viewing	SSQ	Correlates	Heartbeat-evoked potentials as cybersickness correlates
[52]	EEG	19	Projector	Driving (passive)	cont.	Correlates	ICA-based EEG component clusters and sources
[53]	EEG	19	Projector	Driving (passive)	custom	Correlates	Continuous EEG dynamics during driving sickness
[54]	EEG	20	HMD	Other	SSQ	Correlates	ERPs of blur-induced VR visual discomfort
[55]	EEG	16	HMD	Active navigation	SSQ	Correlates	EEG during mobile-VR sickness while walking
[56]	EEG	32	HMD	Mixed	SSQ	Dataset	EEG cybersickness dataset and benchmark
[57]	EEG	32	HMD	Active navigation	SSQ	Classification	EEG-based comparison of VR locomotion techniques
[8]	EEG+ECG+EOG+EGG+ posturography	20	HMD	Active navigation	Likert	Classification	Multimodal EEG+physiology severity classification
[58]	EEG+ECG+GSR	25	Other	Passive viewing	SSQ	Classification	Convolutional-transformer EEG cybersickness detection
[59]	EEG	23	HMD	Passive viewing	SSQ	Classification	Multi-domain EEG feature comparison
[60]	EMG	24	HMD	Passive viewing	MSSQ	Correlates	VEMP changes after VR vection exposure
[61]	GVS	47	HMD	Passive viewing	SSQ	Mitigation	Omnidirectional GVS for 360° video viewing
[62]	EEG+EOG	23	HMD	Passive viewing	SSQ	Classification	LSTM regression of continuous cybersickness severity
[63]	EEG	15	HMD	Mixed	SSQ	Classification	Multi-scale EEG brain-network classification
[64]	EEG	15	HMD	Passive viewing	SSQ	Classification	Bilateral asymmetry features for VRMS detection
[65]	EEG+EOG	22	HMD	Passive viewing	SSQ	Classification	Siam-LSTM with EEG functional brain networks
[66]	EEG+posturography	22	HMD	Active navigation	VRSQ	Classification	Dual-pathway raw EEG and network model
[67]	EEG	40	Screen	Passive viewing	SSQ	Indiv. diff.	EEG band-power correlates of cybersickness susceptibility
[68]	EEG	24	HMD	Passive viewing	cont.	Classification	Deep-learning EEG classification of cybersickness
[69]	EEG	9	Screen	Active navigation	SSQ	Other	Gamma-band EEG visualisation of sickness onset
[70]	EEG+ECG+GSR+EOG	45	CAVE	Active navigation	SSQ	Correlates	EEG and autonomic correlates during navigation
[71]	EEG+ECG+GSR+EOG+ EGG	57	Projector	Active navigation	SSQ	Correlates	Multimodal physiology of navigation cybersickness
[72]	EEG	202	HMD	Passive viewing	Likert	Classification	EEG-to-video transfer for cybersickness prediction
[73]	EEG+EOG	30	HMD	Passive viewing	SSQ	Correlates	Source-localised alpha correlates after VR
[74]	EEG	16	Projector	Driving (passive)	cont.	Classification	ICA-EEG regression of motion-sickness level
[75]	EEG	6	CAVE	Driving (passive)	cont.	Classification	Genetic feature selection for EEG classification
[33]	EEG	43	HMD	Passive viewing	SSQ	Correlates	EEG band-power correlates with continuous SSQ
[76]	EEG+ECG+GSR	40	HMD; Screen	Passive viewing	SSQ	Classification	EEG and physiology fusion for SSQ estimation
[77]	EEG+EOG+posturography	20	Projector	Driving (passive)	cont.	Classification	Multimodal ensemble for VIMS severity
[78]	EEG	18	HMD	Passive viewing	behav.	Classification	Wavelet-packet EEG features for VRMS
[79]	tDCS	20	HMD	Passive viewing	cont.	Mitigation	Cathodal parietal tDCS for cybersickness
[80]	EEG+EOG	22	HMD	Driving (passive)	FMS	Classification	EEG vs. autonomic features for VR sickness
[81]	EEG+EOG	20	HMD	Passive viewing	FMS	Indiv. diff.	EEG biomarker of VRMS resistance

Continued on next page

Table 1 continued from previous page

Ref.	Modality	N	VR device	Stimulus	CS measure	Objective	Topic
[82]	EEG	39	HMD	Mixed	FMS	Classification	Resting EEG prediction of in-car sickness
[83]	EEG+tACS	18	HMD	Mixed	FMS	Mitigation	Online 18 Hz tACS over P3
[84]	EEG	130	HMD	Passive viewing	SSQ	Classification	LSTM prediction of impending cybersickness
[85]	EEG	16	TV	Passive viewing	SSQ	Other	Test-retest reliability of EEG sickness markers
[86]	EEG	9	Projector	Driving (passive)	custom	Correlates	EEG dynamics during VR motion sickness
[87]	EEG	6	CAVE	Driving (passive)	cont.	Classification	BCI estimation of motion-sickness level
[88]	EEG+ECG	25	HMD	Passive viewing	cont.	Classification	Real-time EEG+ECG cybersickness warning
[89]	EEG	9	CAVE	Driving (active)	cont.	Correlates	Wearable EEG and cardiovascular correlates
[90]	EEG	8	Screen	Driving (active)	SSQ	Correlates	Low-channel EEG markers for VIMS
[91]	EEG	36	HMD	Passive viewing	SSQ	Classification	CNN-ECA-LSTM continuous cybersickness regression
[92]	EEG	36	HMD	Passive viewing	SSQ	Classification	EEG spectral and network regression of severity
[93]	EEG	25	HMD	Passive viewing	SSQ	Classification	Wavelet-packet GRU for VR motion sickness
[94]	EEG	9	Screen	Active navigation	SSQ	Classification	Time-domain EEG features for cybersickness
[6]	EEG+EOG	29	HMD	Passive viewing	SSQ	Cognition	P3b ERP attention markers during VR sickness
[95]	EEG+ECG+EOG	8	HMD	Active navigation	custom	Classification	Multimodal symptom-wise cybersickness markers
[96]	EEG	18	HMD	Passive viewing	cont.	Correlates	EEG correlates of display-parameter discomfort
[97]	EEG	40	Screen	Passive viewing	SSQ	Correlates	EEG microstates and functional connectivity
[98]	ECG+GSR	13	HMD	Mixed	VRSQ	Classification	Symbolic ML on peripheral biosignals
[32]	EEG	14	HMD	Passive viewing	SSQ	Correlates	Effective connectivity and predictive coding
[99]	EEG	33	HMD	Passive viewing	SSQ	Correlates	EEG across speed, complexity, and stereoscopy
[100]	EEG	9	Screen	Active navigation	SSQ	Classification	Rule-based EEG severity classification
[101]	GVS+EEG	19	HMD	Active navigation	SSQ	Mitigation	Motion-synchronised GVS during flight VR
[102]	GVS+posturography	140	HMD	Passive viewing	custom	Mitigation	GVS probing of vestibular contribution
[103]	EEG+EMG	28	HMD	Passive viewing	MSSQ	Classification	BioVRSea EEG+EMG sickness prediction
[104]	EEG+EOG	20	HMD	Passive viewing	FMS	Classification	EEG preprocessing trade-offs for cybersickness
[105]	EEG+GSR	22	HMD	Passive viewing	SSQ	Classification	Explainable AI on multimodal physiology
[106]	EEG	25	HMD	Passive viewing	SSQ	Classification	PLV connectivity CNN-LSTM for VIMS
[107]	GVS+GSR	20	HMD	Passive viewing	SSQ	Mitigation	Wearable motion-synchronised GVS
[108]	EEG	22	HMD	Passive viewing	SSQ	Correlates	EEG comparison of HMD versus screen VR
[109]	EEG	31	HMD	Passive viewing	SSQ	Correlates	EEG across VR motion parameter variations
[110]	tDCS+ECG+posturography	20	HMD	Passive viewing	SSQ	Mitigation	Anodal tDCS over right TPJ
[39]	fNIRS+ECG+posturography	11	HMD	Passive viewing	behav.	Correlates	fNIRS and posturography for 3D sickness
[111]	EEG+ECG+EGG	28	HMD	Mixed	SSQ	Indiv. diff.	EEG aperiodic activity for susceptibility profiling
[112]	EEG	53	HMD	Passive viewing	SSQ	Mitigation	Real-time EEG-driven factor-specific mitigation
[113]	EEG	6	HMD	Active navigation	SSQ	Mitigation	Gaze-contingent peripheral blur mitigation
[114]	Other	108	Projector	Mixed	SSQ	Mitigation	Bone-conducted vibration vestibular stimulation
[115]	GVS	40	HMD	Active navigation	SSQ	Mitigation	Noisy GVS for nauseogenic VR
[116]	EEG	6	Projector	Driving (passive)	cont.	Classification	Continuous EEG-based motion-sickness estimation
[117]	EEG	40	TV	Passive viewing	SSQ	Recovery	EEG recovery dynamics after VR exposure
[118]	EEG	17	HMD	Active navigation	SSQ	Cognition	Inhibition-related ERPs after VIMS
[119]	EEG	12	HMD	Other	SSQ	Cognition	Full-body VR exergame and EEG theta
[120]	EEG	16	HMD	Passive viewing	SSQ	Classification	Squeeze-excitation CNN for severity regression
[38]	fNIRS	1	HMD	Active navigation	SSQ	Mitigation	fNIRS-triggered FOV-narrowing mitigation
[37]	fNIRS	10	HMD	Mixed	SSQ	Mitigation	Wearable fNIRS for adaptive FOV restriction
[121]	EEG+ECG	64	HMD	Passive viewing	SSQ	Classification	Interpretable spiking-network EEG prediction
[122]	EEG	25	HMD	Mixed	SSQ	Mitigation	Multisensory mitigation with EEG correlates
[36]	fNIRS	30	HMD	Passive viewing	SSQ	Correlates	fNIRS angular-gyrus correlates of disorientation
[123]	EEG+EOG	15	HMD	Passive viewing	none	Classification	S-transform EEG features for cybersickness

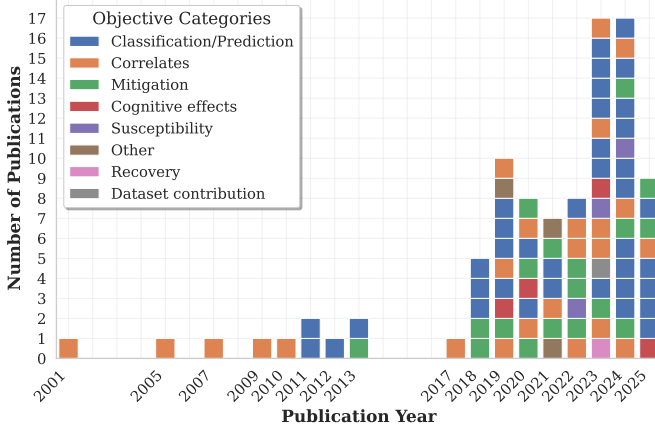


Fig. 2: Distribution of reviewed papers by year and primary research objective.

The remainder of this section synthesises findings thematically along the axes above.

A. Corpus Characteristics

The 92 included studies cluster on EEG-based investigations of young adults in head-mounted VR, but vary widely on baseline condition, montage, and preprocessing. The dispersion documented in this subsection is the precondition for the meta-analytic observations reported in subsection IV-F.

1) *Sample Sizes and Demographics*: The 92 studies span 2001–2025. Across the corpus, 2687 participants were analysed (recruited: 2822), with a mean of 29.2 ± 29.0 participants analysed per study (median: 22, range: 1–202). The 62 studies that report a parsable adult-age range yield a mean age of 23.7 ± 2.7 years, indicating a focus on young-adult populations; 14 studies do not report age or report ranges that cannot be parsed (Figure 3).

2) *Neuroimaging and Physiological Recording Modalities*: EEG is the predominant modality (77 of 92 studies, 83.7%). The remaining studies use fNIRS (4 of 92, 4.3%) or non-invasive neurostimulation without concurrent recording (11 of 92, 12.0%: GVS in 5, tDCS in 3, tACS in 2, and other in 1). No corpus study uses fMRI, MEG, or PET; section III states the search-query and screening implications of this absence.

EEG channel counts vary widely (median 30, range 1–256, mean 39.6 ± 55.8 , $n = 72$ studies reporting). The most frequent montages are 32-channel (18 studies), 64-channel (9), 14-channel (8), 16-channel (5), and 30- or 256-channel (4 each); a long tail of low-channel configurations (4 to 16 channels) is consistent with portable in-VR recording (Figure 4). Channel count is positively correlated with reported within-subject classification accuracy ($r = 0.403$, $p = 0.037$, $n = 27$); this observation is revisited in subsection IV-F.

Concurrent physiological recording is reported in 36 of 92 studies (39.1%), restricted to physiological sensors (electrocardiography, galvanic skin response, electro-oculography, electromyography, electrogastrography, photoplethysmography, respiration, skin temperature, blood pressure); the most common adjuncts are electro-oculography (18 studies), electrocardiography (16), and galvanic skin response (9).

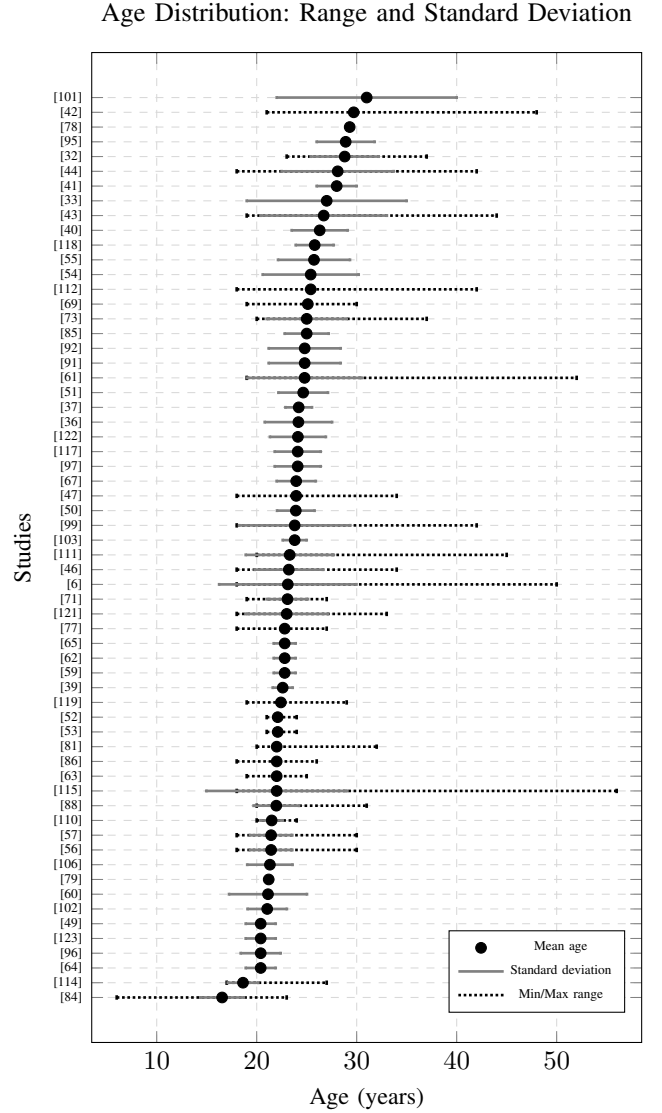


Fig. 3: Age distribution across studies showing mean age (black dots) with age ranges (dotted black error bars) and standard deviations when available (solid gray error bars). The age range shows the minimum and maximum participant ages, while the standard deviation shows the typical spread around the mean age.

3) *Experimental Design and Data Collection Protocols*: Study designs predominantly employ within-subject comparisons (78 of 92, 84.8%), with 7.6% employing between-subject designs exclusively and 7.6% using both. The majority of protocols are single-session (mean 1.6 ± 2.1 sessions, median 1, $n = 91$). Session duration averages 36.3 ± 32.6 minutes (median 25.8, range 1.8–180, $n = 76$), and the cybersickness-inducing stimulus block within a session averages 15.2 ± 15.4 minutes (median 10.0, range 0.5–90, $n = 86$).

Virtual reality is delivered through head-mounted displays in 73.9% of studies (68 of 92), with screens (8.7%) and projection systems (8.7%) as the next most common configurations, followed by CAVE (4.3%), television (2.2%), and other or mixed-modality setups (2.2% combined). Virtual environments

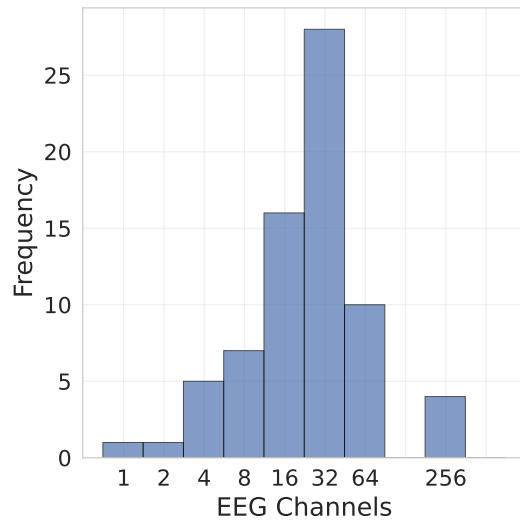


Fig. 4: Distribution of EEG channel configurations across cybersickness studies.

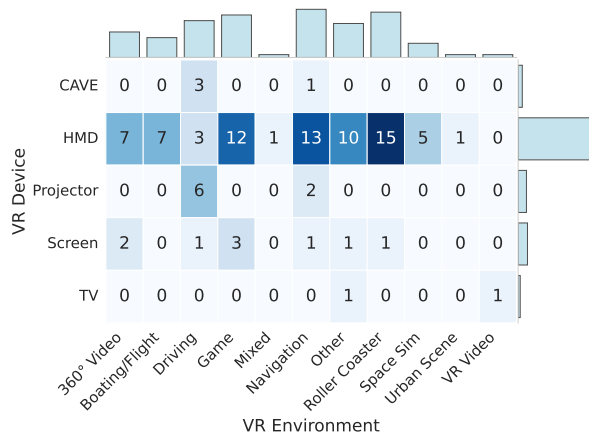


Fig. 5: Heatmap of the distribution of VR device types and virtual environments across studies. Studies employing multiple devices or environments contribute towards each.

cluster on navigation (17 studies, any-mention 18.5%), roller-coaster experiences (16, 17.4%), games (15, 16.3%), driving (13, 14.1%), passive video (10, 10.9%), boating or flight (7, 7.6%), and space simulators (5, 5.4%). The joint distribution of device type and environment is shown in Figure 5; the hardware mix bears on the meta-analytic findings in subsection IV-F.

4) Cybersickness Assessment Methods: Cybersickness is measured through questionnaire instruments in 70 of 92 studies (76.1%), continuous ratings during exposure in 20 (21.7%), single-item Likert scales in 15 (16.3%), and behavioural indices in 8 (8.7%); these categories sum above 100% because 19 studies use more than one method on the same exposure. Among studies using questionnaires, the Simulator Sickness Questionnaire is the most common instrument (59 of 92), followed by the Fast Motion Sickness Scale (6), the Motion Sickness Susceptibility Questionnaire (3), the Virtual Reality Sickness Questionnaire (2), and other instruments (6); 16 stud-

ies do not specify the questionnaire used. Multiple-method use on the same exposure indicates operational-definition variation that constrains cross-study comparability.

5) Baseline Conditions and Preprocessing Heterogeneity: Baseline conditions vary across the corpus: 39 of 92 studies use a task baseline, 16 use an eyes-open resting baseline, 8 use a pre-stimulus segment of the same recording, 5 use an eyes-closed resting baseline, and the remainder either do not specify a baseline (3) or do not report one (21 missing or not applicable, including the neurostimulation-only studies). The five eyes-closed cases are a minority of the corpus, but alpha-band reactivity to eye closure overlaps with the alpha modulations attributed to cybersickness; section V develops this critique and discusses recommended alternatives.

EEG signal preprocessing is heterogeneous in filter selection: band-pass configurations range from 0.01 Hz to 100 Hz with no dominant choice, 25 studies do not specify a band-pass filter, and 72 do not specify a notch filter (Figure 6).

B. Neural and Physiological Correlates of Cybersickness

Across the 77 EEG, 4 fNIRS, and a subset of corpus studies using event-related potential analyses, the corpus converges on a low-frequency power increase during cybersickness; higher-frequency and lateralised findings are less consistent at the corpus level and, as shown in subsection IV-F, are partly attributable to differences in VR hardware platform.

1) EEG Spectral Power: Across the studies reporting per-band changes during cybersickness, the most consistent direction is an increase in low-frequency power. Of the 30 studies reporting a delta-band change, 16 (53.3%) report an increase, 6 a mixed pattern, 5 no change, and 3 a decrease. Of the 35 studies reporting a theta-band change, 20 (57.1%) report an increase, 6 a decrease, 5 a mixed pattern, and 4 no change. Increases in delta (1–4 Hz) and theta (4–8 Hz) power have been associated with the onset and increasing severity of cybersickness across diverse VR stimuli and hardware [32], [33], [43], [67], [71].

Alpha-band findings diverge across the corpus. Of the 39 studies reporting an alpha change, 15 (38.5%) report an increase, 11 (28.2%) report a decrease, 9 (23.1%) report a mixed pattern, and 4 (10.3%) report no change. Studies that report alpha suppression, particularly over central, parietal, and occipital cortices, interpret it as a marker of increased cognitive load under sensory conflict or as a consequence of postural-control demand [26], [53], [70]. Studies that report alpha enhancement attribute it to sensory gating or to drowsiness during prolonged exposure. subsection IV-F shows that the direction of the alpha change tracks VR display hardware, and reframes part of this corpus-level heterogeneity as hardware-attributable.

Higher-frequency bands show less consistent patterns than delta and theta. Of the 36 studies reporting a beta-band change, 11 (30.6%) report an increase, 10 (27.8%) a decrease, 10 a mixed pattern, and 5 no change. Individual reports range from beta suppression [71] to beta enhancement [109], and one study identifies beta as the most informative band for channel selection in classification [124]. Of the 21 studies

Band-pass Filter Frequency Ranges Across Studies

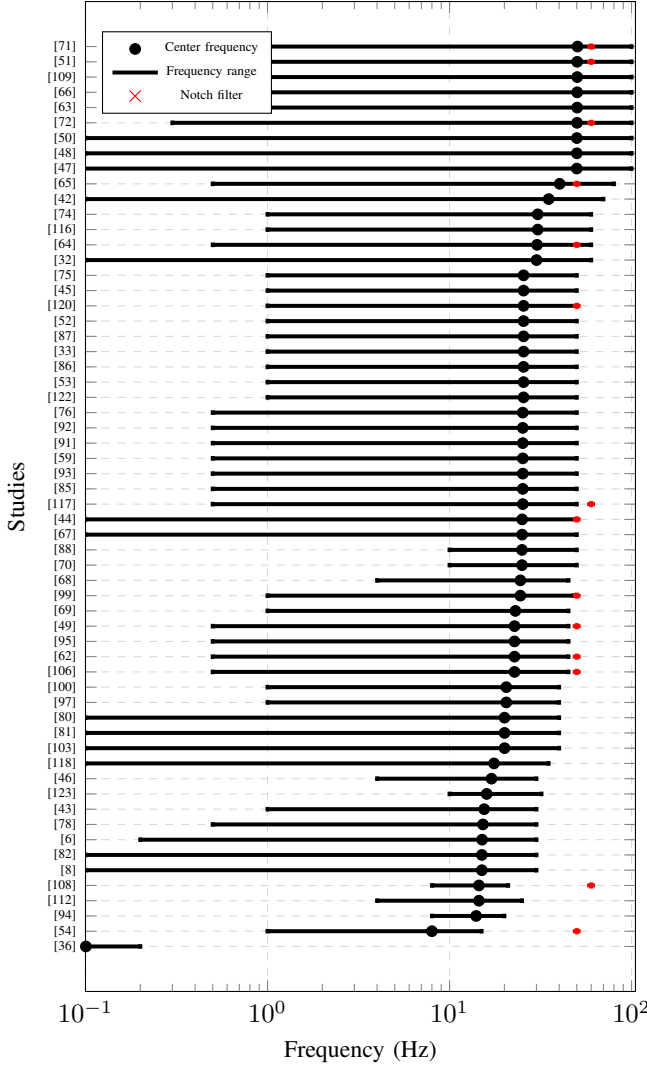


Fig. 6: Band-pass filter frequency ranges across studies showing center frequency (black dots) with frequency ranges (black error bars) and notch filter frequencies (red crosses). The frequency range shows the low and high cutoff frequencies of the band-pass filters, while red crosses indicate notch filter frequencies used to remove specific interference (e.g., 50/60 Hz power line noise).

reporting a gamma-band change, 7 (33.3%) report an increase, 6 no change, 5 a mixed pattern, and 3 a decrease. The full distribution of rhythm changes across anatomical regions is shown in Figure 7; as with alpha, the beta result is partly attributable to display hardware (subsection IV-F).

Test-retest reliability for delta, theta, and alpha power changes in frontal and central areas has been reported in one corpus study [85], and delta-band normalisation has been used as a marker of the post-exposure recovery time course [117]; the recovery literature is discussed in section V.

Beyond band-power, a subset of studies has examined additional spectral and information-theoretic features (entropy, complexity measures, amplitude modulation, mean theta-band

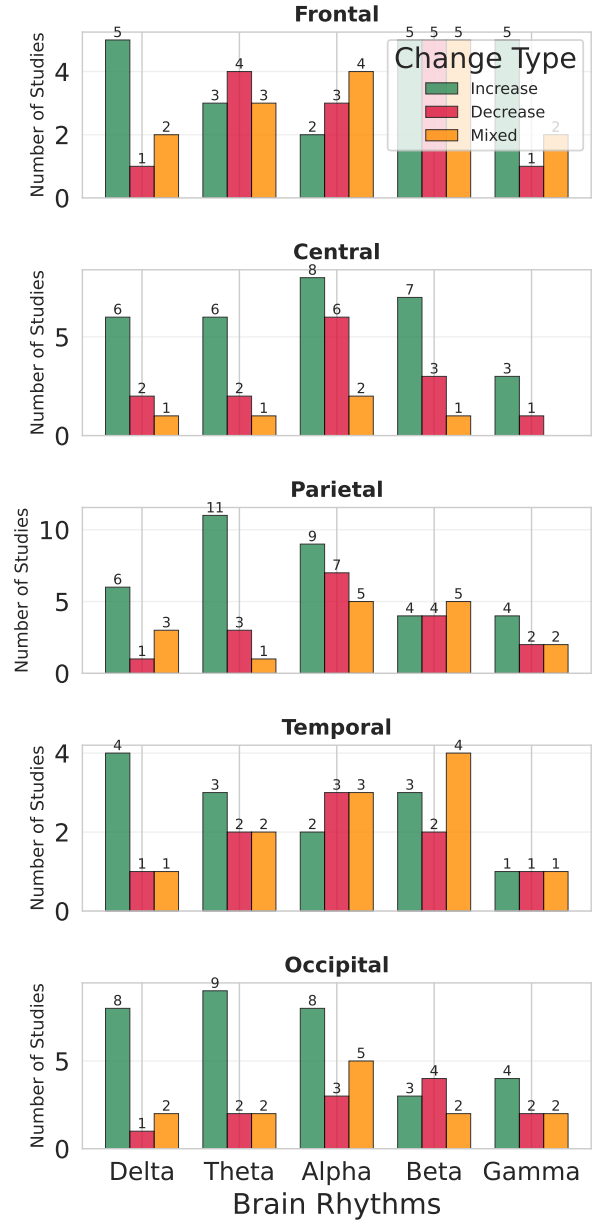


Fig. 7: Brain rhythm changes across anatomical regions during cybersickness.

frequency) [59], [89], [90], [104], [125]. Functional connectivity is investigated in 16 studies with mixed direction (7 mixed, 4 decrease, 3 increase, 2 no change), including transfer-entropy decreases at VIMS onset [32] and reduced connectivity between visual area V5 and vestibular regions [97]; phase-locking-value features have also been used as inputs to deep classification models [66], [106]. The causal relevance of these patterns is supported by intervention studies in which symptom mitigation correlates with normalisation of the same EEG bands [122], [126].

2) *Event-Related Potentials and Evoked Responses:* Beyond ongoing oscillations, a subset of studies has used event-related potentials (ERPs) to characterise how cybersickness affects discrete cognitive processes. These studies indicate

that cybersickness is accompanied by measurable changes in attentional allocation and response inhibition. Cybersickness severity is associated with a reduction in the amplitude of the P3b component in a dual-task paradigm, a classic marker of diminished attentional resources [6]. VIMS has also been shown to impair response inhibition, as reflected by a larger N2 amplitude and a smaller, delayed P3 component during an oddball task [118]. Read alongside the sensory-conflict and cognitive-load accounts introduced in section II, these findings are consistent with cybersickness consuming processing resources that would otherwise support task performance.

Susceptibility-related ERP patterns recur in studies that compare resistant and susceptible participants. VIMS-susceptible participants exhibit excessive parietal N2 amplitudes and impaired theta-band phase synchronisation compared to resistant counterparts [127], and individuals with high sickness levels show higher P3 amplitudes during sensory-mismatch conditions [42]. A separate ERP approach has linked the subjective experience of disorientation to a suppression of fronto-central heartbeat-evoked potentials, providing a link between interoceptive processing and a key symptom of cybersickness [51]. These susceptibility markers are revisited in subsection IV-D alongside the resting-state-prediction literature.

3) *Hemodynamic Responses (fNIRS)*: Four fNIRS studies in the corpus report convergent regional hemodynamics during cybersickness, providing a small but coherent body of evidence on the cortical regions involved. Bilateral angular-gyrus deactivation has been reported in association with subjective sickness scores [36], and increases in total hemoglobin concentration have been linked directly to cybersickness onset [37], [38]. Reduced cortical activation during low-resolution stereoscopic exposure has also been documented [39].

The direction of the hemodynamic effect is not uniform across the small fNIRS literature: regional perfusion increases in some cortical areas coexist with decreases in others, and fNIRS-derived hemoglobin signals have been used as input features for regression models predicting cybersickness severity [128]. Yamamura et al. also demonstrate the only fNIRS-based closed-loop mitigation system in the corpus, which is taken up in subsection IV-E.

C. Classification and Prediction of Cybersickness

Of the 92 corpus studies, 38 have classification or prediction of cybersickness as their primary objective; a broader 41 use classification or regression methods as part of mixed-objective work. Corpus-level claims in this subsection are restricted to the 38-study primary-objective subset. Across these 38 studies, accuracy is the dominant reported performance metric, but sensitivity/specificity, AUC/F1, and the operational definition of the sick/non-sick label are reported in fewer than half of the subset.

1) *Methodological Approaches and Feature Engineering*: Computational approaches across the 38-study subset divide between traditional machine learning (15 of 38, 39.5%), deep learning (14, 36.8%), and regression (6, 15.8%); thresholding (1) and other approaches (2) account for the remainder. Traditional pipelines rely on hand-crafted features (Power Spectral

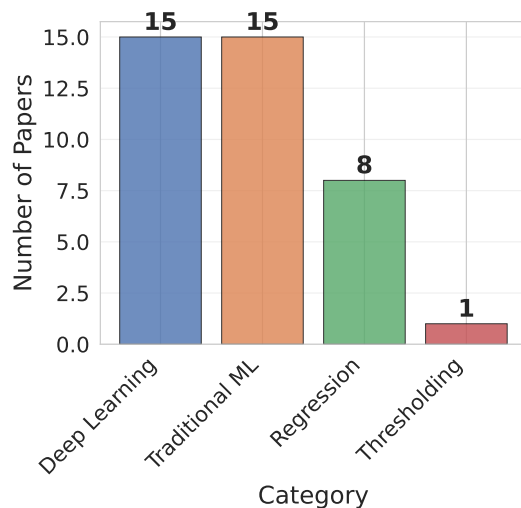


Fig. 8: Distribution of classification method categories across cybersickness studies.

Density, wavelet-derived energy ratios [78], spectral entropy and skew [59], time-domain statistics such as signal variance [94]) fed to Support Vector Machines, k-Nearest Neighbours, or Linear Discriminant Analysis [49], [129]; dimensionality reduction is addressed through Correlation Feature Selection [124] or genetic algorithms [130].

Deep-learning pipelines learn features directly from the data. Convolutional Neural Networks extract spatial and spectral features from EEG time series, spectrograms, or topographic scalp maps [68], with recurrent layers (Long Short-Term Memory [84], Gated Recurrent Units [93]) capturing temporal dynamics. Other architectures include spiking neural networks [121], shallow CNNs designed for real-time detection in mitigation systems [112], hybrid CNN-LSTM models [106], dual-pathway designs that process raw EEG and functional-connectivity inputs in parallel [66], and channel-attention mechanisms [63], [64]. Multimodal fusion combines EEG with peripheral physiological signals [105], with electromyography from postural muscles (reported as more informative than concurrent EEG in one study) [103], or with VR video features via transfer learning [72]; fNIRS regression [128] and explainable-AI interpretation [105] round out the subset.

2) *Reported Performance and Discrimination Metrics*: Model validation predominantly employs k-fold cross-validation (21 of 38, 55.3%), with train-test splits (6), leave-one-subject-out cross-validation (1), and other or unspecified schemes (10) making up the remainder. The distribution of methods and validation schemes is summarised in Figure 8 and Figure 9.

Reported within-subject classification accuracies are high but rest on a small denominator: across the 8 of 38 studies that report a within-subject accuracy, the mean is 90.7% $\pm$ 8.4% (median 91.5%, range 73.3–100.0%). Cross-subject accuracies are reported by 22 of 38 studies, with a mean of 88.1% $\pm$ 11.6% (median 88.9%, range 55.0–99.7%). The two distributions are compared in Figure 10, and the temporal trend

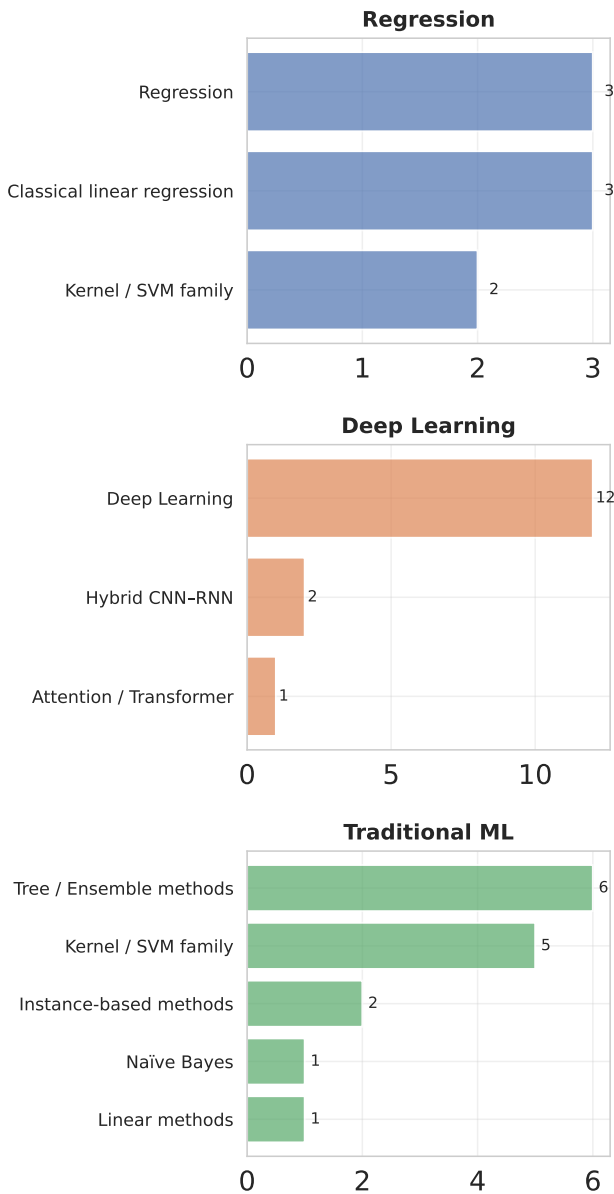


Fig. 9: Detailed breakdown of specific methods within each classification category.

of reported accuracies is shown in Figure 11.

These headline accuracies are reported on heterogeneous discrimination metrics. Of the 38 classification studies, 13 (34.2%) report a headline accuracy with no sensitivity, specificity, AUC, or F1; only 7 (18.4%) report sensitivity and specificity, and 10 (26.3%) report AUC or F1. Ten further studies (26.3%) report only regression metrics (R^2 , RMSE) without a classification accuracy, and 2 (5.3%) report no quantitative performance.

3) *Classification Tasks*: Across the 38-study subset, binary classification of a “sick” versus “not sick” state is the most common task. Several studies report within-subject accuracies above 95% on this binary task [57], [63], [64], [68], [106], [129]. The sick/non-sick label that grounds these accuracies varies in how it is operationalised: 19 of 38 classification

studies expose the threshold or its basis, of which 8 use a numeric cutoff and the remainder use self-reported binary labels, pre/post differences, total-questionnaire-score cutoffs, or do not specify; the other 19 studies do not document the threshold at all.

A second task is multi-level classification, which categorises cybersickness into several discrete levels of severity. Three-level classification has been demonstrated with multimodal fusion [8], [77] and with EEG alone using rule-based induction [100] or a genetic algorithm combined with a Support Vector Machine [87].

A third task is continuous prediction or regression of a cybersickness severity score, measured by correlation coefficients and error metrics. Deep-learning models have demonstrated high correlations between predicted and actual SSQ scores [62], [65], [76], [91], [92], [120], while earlier work used Support Vector Regression [74], [116] and linear regression [131]. Addressing the threshold-heterogeneity problem raised above, one recent study develops continuous, calibration-free detection directly from physiological data with dynamic real-time ground-truth labels [132].

D. Individual Differences and Susceptibility

Resting-state neural features have been used to predict cybersickness susceptibility before exposure, and during exposure susceptible and resistant individuals show distinct neural signatures. Demographic moderators are reported in a small subset of the corpus and warrant cautious interpretation given the small effect sizes documented in the broader literature.

Pre-exposure prediction of cybersickness has been demonstrated in three corpus studies. A spiking neural network classified subsequent sickness with 85.9% accuracy from pre-exposure resting-state EEG alone [121]. Resting-state EEG beta power in the left parietal region has been reported as a significant predictor of subsequent motion-sickness ratings in an in-car VR platform [82], and aperiodic components of the EEG signal have been reported as predictive of susceptibility to rotational motion [111]. These findings point to pre-existing neural traits that influence susceptibility, although the evidence base remains small.

During cybersickness exposure, resistant and susceptible individuals show distinct neural signatures. Resistant individuals exhibit enhanced theta activity in the left parietal cortex, a candidate biomarker of resilience [81]. Susceptible individuals show excessive parietal N2 ERP amplitudes and impaired theta-band phase synchronisation [127], higher P3 amplitudes during sensory-mismatch conditions [42], and differential spectral and source-space activation patterns duringvection [133]. One study reports that the cybersickness condition produces increased delta and decreased alpha power in both susceptible and non-susceptible groups, but susceptible participants additionally show lower absolute power in the beta and gamma bands at baseline [67]. An earlier study using a long-duration driving task found that the ratio of theta to total EEG power in frontal and parietal lobes differed between sick and non-sick groups, proposing this ratio as a physiological index of sensitivity [134].

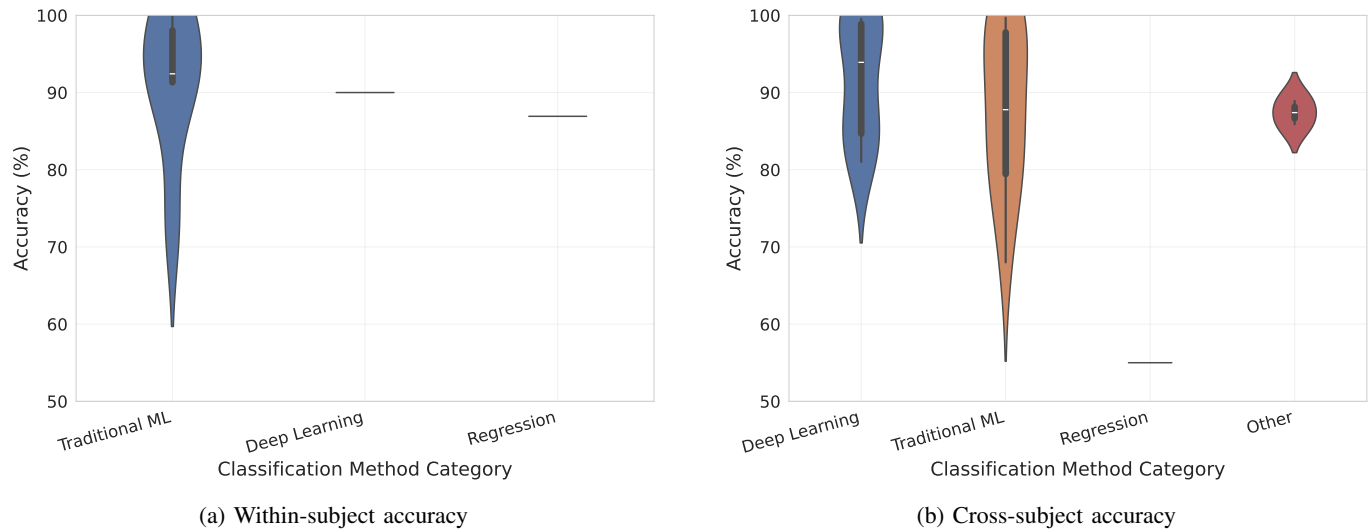


Fig. 10: Classification accuracy distributions by method category for within-subject and cross-subject validation approaches.

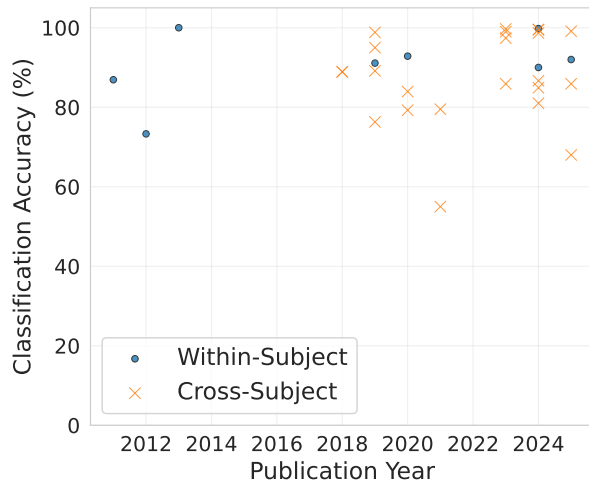


Fig. 11: Classification accuracy over time, comparing within-subject and cross-subject validation approaches.

Sex and gender differences are reported in 3 of 92 corpus studies, all from a single laboratory using a VR-neurofeedback paradigm [46]–[48]; an additional 11 corpus studies use male-only samples (precluding sex comparisons) and 15 do not report participant sex composition, leaving 63 mixed-sex studies that report composition without analysing sex effects. The broader cybersickness literature outside the corpus calibrates these findings. A recent meta-analytic synthesis concludes that women experience greater cybersickness than men, but that the effect size is modest (meta-analytic $r \approx 0.21$) and that the field is widely underpowered, requiring approximately 176 participants per arm for 80% statistical power to reliably detect the effect [135]. A separate review of 76 studies finds that only 37 of those studies (48.7%) are consistent with the assertion that women are more susceptible than men [136]. In a single experimental study, the difference between male and female participants did not reach significance when gaming experience was included as a covariate [137]. Read alongside

this broader literature, the 3 of 92 corpus reports are consistent with an underpowered signal of a small effect that gaming experience and other moderators may partially explain, rather than with a corpus-wide sex difference.

Individual variability in cybersickness susceptibility has measurable neural correlates and small demographic moderators. At present, the corpus is underpowered for either to be operationalised as a predictive biomarker in a deployment setting. The mitigation literature reviewed next acts on the neural signals identified here.

E. Mitigation and Intervention Strategies

Mitigation research in the corpus splits into two threads: closed-loop adaptive VR that modifies the virtual environment in response to detected sickness (2 corpus studies), and interventions that target the user’s brain state directly (5 corpus studies including transcutaneous auricular vagus nerve stimulation, vibro-motor reprocessing therapy, multisensory cueing, and neurofeedback). Both threads are early-stage, and the small literature already documents one methodological obstacle specific to the closed-loop setting.

1) *Closed-Loop Adaptive VR*: Only two corpus studies implement closed-loop adaptation: the literature is in early-stage development. One system uses a two-stage shallow convolutional network to detect cybersickness from EEG and infer a likely causal factor, then selectively modifies the implicated VR parameter (navigation speed or scene complexity), producing a significant reduction in user-reported sickness compared to a control condition [112]. A second system uses fNIRS to monitor total hemoglobin concentration; on detecting increases correlated with sickness, the system dynamically reduces the user’s field of view, demonstrating the feasibility of an fNIRS-based closed-loop pipeline [37].

2) *Interventions Modulating Brain State*: Five corpus studies investigate interventions intended to act on the underlying physiological state of the user, with concurrent EEG used to verify neural engagement. Transcutaneous auricular

vagus nerve stimulation significantly reduced motion-sickness symptoms in one study, with the reduction correlated with stimulation-induced neural activation in specific brain regions measured by EEG source localisation [138]. Vibro-motor reprocessing therapy was associated with significant reductions in alpha, delta, and theta spectral power [126]. Providing synchronised auditory and physical motion cues during a simulated bicycle ride reduced VIMS scores relative to conditions with mismatched or absent cues, with the effect reflected in concurrent EEG recordings [122].

Two further studies, both from the same laboratory, investigated neurofeedback protocols in which users were trained to volitionally modulate their own brain activity. The presence of cybersickness symptoms was found to directly impair the user's ability to successfully perform the neurofeedback training task [48], and a placebo transcranial direct current stimulation procedure — while having no significant effect on reported sickness symptoms — paradoxically degraded performance on a concurrent neurofeedback task [47]. These findings illustrate a methodological challenge for any neuromarker-driven closed-loop intervention: the physiological state being targeted can interfere with the user's ability to engage with the intervention itself.

F. Corpus-Level Methodological Observations

Meta-analytic comparison across the 92 corpus studies reveals that VR display hardware substantially modulates EEG band-power responses and that stimulus-exposure duration is confounded with hardware. The findings in this subsection are observational; the interpretive critique and reporting recommendations they motivate are developed in section V.

Alpha-band cybersickness responses differ between head-mounted-display and screen-based studies. A Mann-Whitney U test on the direction of alpha change between HMD studies ($n = 24$ reporting) and screen-based studies ($n = 4$) yields Cliff's $\delta = 0.875$ ($p = 0.004$), a large effect. The underlying distribution is asymmetric: HMD studies report alpha increases four times as often as decreases (12 of 24 versus 3 of 24), whereas screen-based and other non-HMD studies report alpha decreases nearly three times as often as increases (8 of 15 versus 3 of 15). This pattern reframes the alpha-band heterogeneity reported in subsection IV-B as partly hardware-attributable, rather than as a pure intersubject or task effect.

The same comparison for the beta band yields Cliff's $\delta = 0.664$ ($p = 0.015$, large effect; HMD $n = 25$, screen $n = 5$), indicating that the hardware effect extends beyond alpha. A Kruskal-Wallis test across the full VR-device categorisation yields $\eta^2 = 0.251$ ($p = 0.009$) for alpha and $\eta^2 = 0.160$ ($p = 0.033$) for beta, corroborating the binary hardware comparison. The hardware effect does not extend to delta, theta, or gamma, which show no significant between-device differences; the claim is therefore bounded to higher-frequency bands.

Stimulus-exposure duration is itself confounded with hardware. HMD studies use shorter exposure periods (mean 12.8 ± 14.6 minutes, $n = 64$) than non-HMD studies (mean 22.1 ± 15.9 minutes, $n = 22$); Mann-Whitney $U = 412.5$, $p = 0.004$,

Cliff's $\delta = -0.414$ (medium effect). The hardware confound therefore covaries with how long the user is exposed, not only with the display optics; comparisons of band-power changes between HMD and screen studies necessarily mix the two effects.

These corpus-level observations, together with the inline reporting-gap counts in subsection IV-C and the baseline-heterogeneity findings in subsection IV-A, seed the design and reporting recommendations developed in section V.

V. DISCUSSION

The prior scoping review of cybersickness EEG research [1] reported delta-band increases, beta-band decreases, and inconsistent alpha-band findings; the present corpus refines this picture in two respects. The delta- and theta-band increases reproduce in the current corpus (16 of 30 studies for delta; 20 of 35 for theta). The reported alpha-band inconsistency partly resolves once VR display hardware is accounted for: head-mounted-display and screen-based studies differ with large effects on alpha (Cliff's $\delta = 0.875$, $p = 0.004$) and beta ($\delta = 0.664$, $p = 0.015$); see subsection IV-F.

One corpus study documents post-exposure neural dynamics, reporting delta-band normalisation following VR exposure [117]. The corpus does not yet link these residual signatures to behaviour, and longitudinal designs are recommended in subsection V-C.

A. Objective Markers and the Limits of Self-Report

Cybersickness has no diagnostic biomarker; ground truth is self-report. Four corpus studies report functional cognitive consequences of this state: working-memory decrements scaling with reported severity [6] and inhibitory-control demand reflected in N2/P3 components [118] (see subsection IV-B), motivating objective markers that do not depend on self-report. Of these candidate markers, postural sway has a direct theoretical anchor in the postural-instability account, which predicts that pre-symptomatic sway covaries with subsequent severity [24], [25]; VR-specific reports document moderate coupling [8], [28], [139]. Pupillometry is a second candidate [137], [140], subject to arousal and cognitive-load confounds. Sixteen of the 92 corpus studies already co-record force-plate, balance-board, eye-tracker, or pupillometry alongside EEG, but typically as covariates rather than as candidate ground truth.

B. Methodological Challenges and Critical Perspective

Four classes of methodological issue emerge from the corpus and constrain cross-study synthesis in cybersickness neuroimaging and neurostimulation: (1) classification reporting practices, (2) baseline selection, (3) protocol and hardware heterogeneity, and (4) conceptual and terminological variability.

Across the 38 classification or prediction studies in the corpus, three reporting practices limit the interpretability of headline performance numbers. Thirteen studies report a headline accuracy without sensitivity, specificity, AUC, or

F1; under class imbalance, accuracy collapses toward the majority-class baseline and provides little information about the classifier's behaviour at decision-relevant operating points [141], [142]. Nineteen studies expose no sick-versus-non-sick threshold (neither a numeric cutoff nor a stated basis), and only eight give an explicit numeric cutoff converting SSQ or FMS scores into a label; the target variable is therefore operator-defined and not comparable across studies. One study reports leave-one-subject-out validation; twenty-one use k-fold cross-validation without subject stratification, which permits subject-identity leakage from training into the test fold and inflates apparent generalisation [143]. These three gaps are consistent with the high headline accuracies recurring in the corpus, including binary accuracies above 95% [57], [63], [64], [66], [106] and pre-post resting-state contrasts approaching 99% [49]: where accuracy is the only reported metric, the threshold defining the label is undocumented, and the validation scheme does not respect subject identity, the headline number does not support a generalisation claim. A related ground-truth issue compounds these reporting gaps: the SSQ is retrospective and coarse, and continuous in-task ratings risk encoding the motor act of reporting rather than the sickness state itself [104], so the very signal that supervises classification is heterogeneous before any modelling choice is made.

A second design issue concerns the baseline against which in-exposure neural change is measured. Closing the eyes elicits a large posterior alpha increase relative to eyes-open [144]–[146], so a study that contrasts VR exposure against an eyes-closed pre-task recording confounds the alpha-change estimate with a state shift unrelated to the visual stimulus. The VR exposure condition is, by construction, eyes-open inside a head-mounted display viewing a structured scene; an eyes-closed pre-task block is therefore not matched to the exposure condition on visual input, oculomotor state, or vigilance. This baseline choice is a minority practice in the corpus (5 of 77 EEG studies, or 6 if eyes-open + eyes-closed composites are counted), but distorts the alpha-band estimates of the affected studies, which typically present those estimates as primary outcomes.

Baseline choice is one of several protocol parameters that vary widely across the corpus, and the broader heterogeneity carries methodological consequences. At the corpus level, VR display hardware substantially modulates the band-power response: head-mounted-display and screen-based studies differ on the direction of alpha change with a large effect (Cliff's $\delta = 0.875$, $p = 0.004$) and on beta change with a medium-to-large effect ($\delta = 0.664$, $p = 0.015$), and the alpha-band inconsistency reported in prior reviews [1] partly resolves once hardware is accounted for (see subsection IV-F). Stimulus exposure duration is itself confounded with display: head-mounted-display studies use shorter exposures (12.8 minutes on average, $n = 64$) than non-head-mounted-display studies (22.1 minutes, $n = 22$; Cliff's $\delta = -0.414$, $p = 0.004$), so comparisons of band-power change between display classes necessarily mix the two effects. Recording quality varies similarly, ranging from research-grade systems to low-cost consumer-grade devices with fewer channels and lower signal quality [90], and the corpus is biased toward low-density montages: among

the 72 EEG studies that disclose channel count, 58 (80.6%) use 32 or fewer channels and 14 (19.4%) use 10 or fewer. Low-density montages are appropriate for applied paradigms that target well-characterised localised signals; for exploratory characterisation of cybersickness neurophysiology they have insufficient spatial resolution for reliable source localisation. Reporting quality compounds the hardware-protocol heterogeneity: of the 77 EEG corpus studies, 43 (55.8%) do not state the reference electrode and 59 (76.6%) do not state a notch-filter frequency, which limits both reproducibility and meta-analytic re-pooling. Samples are small and demographically narrow (typically young university students), and 11 of the 92 studies recruited male-only samples [67], [126], which limits statistical power for individual-difference questions including the sex/gender effects discussed in subsection V-C. Finally, the experiences and stimuli are themselves heterogeneous: passive 360-degree video [109] is not equivalent to active interactive navigation [26], and findings from head-mounted-display, screen, CAVE, and smartphone-VR systems [85], [126], [147] are often discussed interchangeably.

Finally, the field exhibits conceptual variability in both the use of theory and the use of terminology. Most corpus studies (73 of 92 explicitly, 14 implicitly) cite an etiological theory of cybersickness in their introduction or methods, but few return to that framework when interpreting their neurophysiological findings; a neural feature observed in a given paradigm could reflect sensory conflict, postural control, expectation-related vection, display optics, or some combination, and without an explicit theoretical commitment the literature cannot adjudicate between these possibilities. Terminology compounds the interpretive ambiguity: motion sickness, visually induced motion sickness, simulator sickness, and cybersickness are related but non-identical constructs tied to different exposure contexts (see section II), yet they are frequently used interchangeably [11], [12], [14], [15]. A similar conflation arises between sickness symptoms and the perceptual phenomenon of vection, despite the latter being absent from the SSQ and from the Bárány diagnostic criteria [7], [12], [17]. A further consequence of these practices is the labelling of any statistically significant feature as a “neuromarker” without establishing its specificity, sensitivity, or reliability across contexts.

Taken together, these methodological and conceptual issues constrain cross-study synthesis and reproducibility; the corresponding forward directions are developed in subsection V-C.

C. Future Directions

The methodological challenges documented in section V translate into six concrete directions for future work: standardised reporting and open data, causal mechanisms, generalisation and ecological validation, sex- and gender-stratified design, objective behavioural markers, and longitudinal characterisation of adaptation and recovery.

First, the field needs a standardised reporting set and shared infrastructure. For classification and prediction studies, a minimum reporting set follows from the gaps documented in section V: a primary discriminative metric (AUC or balanced accuracy) reported with a 95% confidence interval; sensitivity

and specificity at a stated operating point; the class balance of the test fold; the numeric SSQ or FMS cutoff that converts continuous ratings into a label; a validation scheme that respects subject identity (leave-one-subject-out or held-out-subject k-fold); and a comparison against a trivial baseline (majority class, chance) and at least one non-neural feature baseline. For studies that quantify in-exposure neural change, the baseline used to anchor the contrast should be matched to the exposure condition on visual input, oculomotor state, and vigilance: an eyes-open resting state with the head-mounted display on and a static neutral scene [82], a pre-stimulus segment immediately preceding stimulus onset [118], or a sham non-nauseogenic VR exposure of matched duration [110]; the choice should be reported with its rationale. For EEG, the reference electrode and notch-filter frequency should be stated (currently absent from 56% and 77% of the 77 EEG corpus studies, respectively); both are required for reproducibility and meta-analytic re-pooling. These reporting standards complement, rather than replace, the call for shared infrastructure: community-curated benchmark datasets that span passive (roller-coaster) and active (navigation) experiences, with multimodal recordings (EEG, EOG, ECG, head motion, sway, eye tracking) and well-defined subjective labels. A small number of corpus papers have already released public datasets [56], [72], [132]; releasing analysis code and pre-trained models alongside the data would further support replication and cumulative analysis.

Second, the field should move beyond correlation toward causal mechanisms. The literature has documented associations between specific neural patterns and cybersickness across multiple studies, but the causal status of these associations remains largely unexamined. Non-invasive brain stimulation offers a direct test: transcranial alternating-current stimulation can target specific oscillations (e.g., alpha or theta), and existing corpus studies that combine stimulation with VR exposure [47], [110] provide a methodological template for asking whether modulating a candidate neuromarker changes sickness severity. Coupled with computational models of multisensory integration and predictive coding, interventional designs can begin to distinguish neural features that drive the cybersickness experience from features that merely accompany it.

Third, models built for cybersickness must generalise and must be validated in ecologically valid contexts. In the four corpus studies that report both within- and cross-subject accuracy, cross-subject performance is lower in each case, and only one of 38 classification studies reports a leave-one-subject-out evaluation; transfer-learning and domain-adaptation approaches that explicitly model inter-subject and inter-session variability are necessary if brain-directed methods are to operate without per-user calibration. Validation should also move beyond the laboratory: at-home, naturalistic paradigms such as the in-home gaming setup of [148] are needed to assess whether laboratory neuromarkers survive the noise of consumer-grade hardware and uncontrolled environments.

Fourth, sex and gender should be analysed as a primary factor. Of the 92 corpus studies, 11 used male-only samples, 15 did not report sex composition, and only three – all from a single laboratory, and therefore not independently replicated – reported higher cybersickness in women [46]–[48]. The

broader literature on sex differences in cybersickness is mixed: [135] argues that reported effects are modest in magnitude and plausibly mediated by interpupillary-distance fit, prior gaming experience, and questionnaire framing; [137] reports that male–female differences disappear when prior gaming experience is included as a covariate; and [136] caution that the claim of greater female susceptibility is overstated in the simulation literature. Sex-stratified outcomes (band-power changes, connectivity, classification metrics) should be reported even when single-study samples are underpowered for inferential testing, so that subsequent meta-analytic re-pooling is possible. Interpupillary-distance setting and gaming experience should be logged as covariates, both because they confound sex effects and because they connect to the hardware effects documented in subsection IV-F. Adequately powered designs — pooled multi-site cohorts or longitudinal within-subject designs — are needed before any individual study can credibly settle the magnitude or moderators of a sex effect on cybersickness.

Fifth, the field should develop objective behavioural markers to supplement self-report. The candidate markers introduced in section V are already being collected by 16 of the 92 corpus studies, but typically as covariates rather than as candidate ground truth. Postural sway is the strongest theoretically anchored candidate: studies should report a coupling estimate between pre-symptomatic sway dynamics and subsequent self-reported sickness, alongside the protocol metadata that this estimate depends on (stance, surface, sampling rate). Pupilometry can supplement self-report subject to arousal and cognitive-load confounds, and requires controls on ambient luminance and on task demand. Heart-rate variability, electrodermal activity, and head-motion dynamics are further candidates whose corpus-level predictive value is not yet quantified; multi-modal integration that fuses behavioural, autonomic, and neural signals offers a more holistic state estimate than any single modality. Adopting these markers as outcome variables, rather than as covariates, is the precondition for shifting cybersickness research toward objective ground truth.

Finally, the field should adopt longitudinal study designs to characterise adaptation and recovery. The corpus is dominated by single-session experiments, leaving the neurodynamics by which users adapt to VR (“getting one’s VR legs”) largely uncharacterised, and the single corpus recovery study [117] is insufficient to support strong claims about residual neural state. Multi-session within-subject designs are needed to track how the EEG signatures identified in this review evolve with repeated exposure, and to map the dissipation of post-exposure signatures onto behaviour.

VI. CONCLUSION

The adverse effects of cybersickness remain a significant obstacle to the widespread adoption and prolonged use of Virtual Reality. Brain-Computer Interfaces have emerged as a promising approach for moving beyond subjective reports toward more objective measurement, prediction, and potential mitigation of this debilitating condition. This paper has provided a systematic and critical review of the literature at

the intersection of BCI and cybersickness, synthesizing the current state of knowledge and identifying key challenges and opportunities.

Our synthesis suggests an emerging neurophysiological signature of cybersickness, primarily characterized by increased low-frequency (delta, theta) power and a suppression of alpha-band oscillations in the EEG. Spatially precise modalities like fMRI and fNIRS have localized these dynamics to a network of brain regions including the insula, cingulate cortex, and areas involved in visual-vestibular processing. This foundational knowledge has facilitated the development of computational models, many based on deep learning, that show promise in classifying sickness states, though accuracy claims require careful interpretation given methodological limitations. This research has led to the development of early-stage, real-time, closed-loop adaptive systems that attempt to use neural feedback to modify the VR environment and potentially reduce user discomfort.

However, this review also highlights that the field is hampered by significant methodological heterogeneity, a lack of standardized protocols and ground-truth labels, and a frequent reliance on small, demographically-limited samples. These challenges severely limit the generalizability and replicability of many findings and impede the direct comparison of results across studies. The path toward robust, generalizable BCI applications for cybersickness management requires a concerted shift toward open science practices, including the development of benchmark datasets and standardized evaluation metrics. It also necessitates a move from purely correlational findings to causal investigations and the validation of systems in ecologically relevant, real-world contexts.

In conclusion, the integration of BCI with VR represents more than a novel tool for studying cybersickness; it is a critical enabling technology for the future of immersive computing. The development of truly user-aware systems that can implicitly monitor and dynamically adapt to a user's neurophysiological state in real-time may help address one of the most persistent barriers to the widespread adoption of VR. This line of research is essential for paving the way for experiences that are not only more immersive and effective, but also universally comfortable and accessible.

ACKNOWLEDGMENTS

This research was funded by the European Union ERANET CHIST-ERA 2020 (CHIST-ERA-20-BCI-003 - "GENESIS") and the French Research Agency (ANR-21-CHRA-0001-01).

REFERENCES

- [1] E. Chang, M. Billingham, and B. Yoo, "Brain activity during cybersickness: A scoping review," *Virtual Reality*, vol. 27, no. 3, pp. 2073–2097, Sep. 2023.
- [2] J. Chatain, V. Ramp, V. Gashaj, V. Fayolle, M. Kapur, R. W. Sumner, and S. Magnenat, "Grasping Derivatives: Teaching Mathematics through Embodied Interactions using Tablets and Virtual Reality," in *Proceedings of the 21st Annual ACM Interaction Design and Children Conference*, ser. IDC '22. New York, NY, USA: Association for Computing Machinery, Jun. 2022, pp. 98–108.
- [3] S. S. Kardong-Edgren, S. L. Farra, G. Alinier, and H. M. Young, "A Call to Unify Definitions of Virtual Reality," *Clinical Simulation in Nursing*, vol. 31, pp. 28–34, Jun. 2019.
- [4] P. Caserman, A. Garcia-Agundez, A. Gámez Zerban, and S. Göbel, "Cybersickness in current-generation virtual reality head-mounted displays: Systematic review and outlook," *Virtual Reality*, vol. 25, no. 4, pp. 1153–1170, Dec. 2021.
- [5] S. Weech, S. Kenny, and M. Barnett-Cowan, "Presence and Cybersickness in Virtual Reality Are Negatively Related: A Review," *Frontiers in Psychology*, vol. 10, 2019.
- [6] K. J. Mimnaugh, E. G. Center, M. Suomalainen, I. Becerra, E. Lozano, R. Murrieta-Cid, T. Ojala, S. M. LaValle, and K. D. Federmeier, "Virtual Reality Sickness Reduces Attention During Immersive Experiences," *IEEE Transactions on Visualization and Computer Graphics*, vol. 29, no. 11, pp. 4394–4404, Nov. 2023.
- [7] R. S. Kennedy, N. E. Lane, K. S. Berbaum, and M. G. Lilienthal, "Simulator Sickness Questionnaire: An Enhanced Method for Quantifying Simulator Sickness," *International Journal of Aviation Psychology*, vol. 3, no. 3, p. 203, Jul. 1993.
- [8] M. S. Dennison, M. D'Zmura, A. V. Harrison, M. Lee, and A. J. Raglin, "Improving motion sickness severity classification through multi-modal data fusion," in *Artificial Intelligence and Machine Learning for Multi-Domain Operations Applications*, T. Pham, Ed. Baltimore, United States: SPIE, May 2019, p. 27.
- [9] J. R. Lackner, "Motion sickness: More than nausea and vomiting," *Experimental Brain Research*, vol. 232, no. 8, pp. 2493–2510, Aug. 2014.
- [10] B. Keshavarz and J. F. Golding, "Motion sickness: Current concepts and management," *Current Opinion in Neurology*, vol. 35, no. 1, pp. 107–112, Feb. 2022.
- [11] R. S. Kennedy, J. Drexler, and R. C. Kennedy, "Research in visually induced motion sickness," *Applied Ergonomics*, vol. 41, no. 4, pp. 494–503, Jul. 2010.
- [12] Y.-H. Cha, J. F. Golding, B. Keshavarz, J. Furman, J.-S. Kim, J. A. Lopez-Escamez, M. Magnusson, B. J. Yates, and B. D. Lawson, "Motion sickness diagnostic criteria: Consensus Document of the Classification Committee of the Bárány Society," *Journal of Vestibular Research*, vol. 31, no. 5, pp. 327–344.
- [13] B. Keshavarz, H. Hecht, and B. D. Lawson, "Visually Induced Motion Sickness: Causes, Characteristics, and Countermeasures," in *Handbook of Virtual Environments: Design, Implementation, and Applications, Second Edition*, 2014, vol. 34, pp. 1–3.
- [14] K. M. Stanney, R. S. Kennedy, and J. M. Drexler, "Cybersickness is Not Simulator Sickness," *Proceedings of the Human Factors and Ergonomics Society Annual Meeting*, vol. 41, no. 2, pp. 1138–1142, Oct. 1997.
- [15] L. Rebenitsch and C. Owen, "Review on cybersickness in applications and visual displays," *Virtual Reality*, vol. 20, no. 2, pp. 101–125, Jun. 2016.
- [16] B. Keshavarz and H. Hecht, "Validating an efficient method to quantify motion sickness," *Human Factors*, vol. 53, no. 4, pp. 415–426, Aug. 2011.
- [17] L. J. Hettinger, K. S. Berbaum, R. S. Kennedy, W. P. Dunlap, and M. D. Nolan, "Vection and Simulator Sickness," *Military Psychology*, vol. 2, no. 3, pp. 171–181, Sep. 1990.
- [18] B. Keshavarz, B. E. Riecke, L. J. Hettinger, and J. L. Campos, "Vection and visually induced motion sickness: How are they related?" *Frontiers in Psychology*, vol. 6, 2015.
- [19] J. T. Reason, J. T. Reason, J. J. Brand, and J. J. Brand, *Motion Sickness*. London: Acad. Pr, 1975.
- [20] J. T. Reason, "Motion Sickness Adaptation: A Neural Mismatch Model," *Journal of the Royal Society of Medicine*, vol. 71, no. 11, pp. 819–829, Nov. 1978.
- [21] C. M. Oman, "Motion sickness: A synthesis and evaluation of the sensory conflict theory," *Canadian Journal of Physiology and Pharmacology*, vol. 68, no. 2, pp. 294–303, Feb. 1990.
- [22] W. Bles, J. E. Bos, B. De Graaf, E. Groen, and A. H. Wertheim, "Motion sickness: Only one provocative conflict?" *Brain Research Bulletin*, vol. 47, no. 5, pp. 481–487, Nov. 1998.
- [23] J. Prothero and D. Parker, "A unified approach to presence and motion sickness," in *Virtual and Adaptive Environments: Applications, Implications, and Human Performance Issues*. Mahwah, NJ, US: Lawrence Erlbaum Associates Publishers, 2003, pp. 47–66.
- [24] G. E. Riccio and T. A. Stoffregen, "An ecological Theory of Motion Sickness and Postural Instability," *Ecological Psychology*, vol. 3, no. 3, pp. 195–240, Sep. 1991.
- [25] T. A. Stoffregen and L. Smart, "Postural instability precedes motion sickness," *Brain Research Bulletin*, vol. 47, no. 5, pp. 437–448, Nov. 1998.

- [26] C. A. T. Cortes, C.-T. Lin, T.-T. N. Do, and H.-T. Chen, "An EEG-based Experiment on VR Sickness and Postural Instability While Walking in Virtual Environments," Feb. 2023.
- [27] J. Teixeira, S. Miellet, and S. Palmisano, "Unexpected Vection Exacerbates Cybersickness During HMD-Based Virtual Reality," *Frontiers in Virtual Reality*, vol. 3, Apr. 2022.
- [28] —, "Investigating the Relative Contributions of Unexpected Vection and Postural Instability to VR Cybersickness," *International Journal of Human-Computer Interaction*, pp. 1–17, Apr. 2024.
- [29] G. Kramida, "Resolving the Vergence-Accommodation Conflict in Head-Mounted Displays," *IEEE Transactions on Visualization and Computer Graphics*, vol. 22, no. 7, pp. 1912–1931, Jul. 2016.
- [30] S. M. Ebenholtz, "Motion sickness and oculomotor systems in virtual environments," *Presence: Teleoper. Virtual Environ.*, vol. 1, no. 3, pp. 302–305, Jan. 1992.
- [31] M. Treisman, "Motion Sickness: An Evolutionary Hypothesis," *Science*, vol. 197, no. 4302, pp. 493–495, Jul. 1977.
- [32] M. Nürnberg, C. Klingner, O. W. Witte, and S. Brodoehl, "Mismatch of Visual-Vestibular Information in Virtual Reality: Is Motion Sickness Part of the Brains Attempt to Reduce the Prediction Error?" *Frontiers in Human Neuroscience*, vol. 15, p. 757735, Oct. 2021.
- [33] E. Krokos and A. Varshney, "Quantifying VR cybersickness using EEG," *Virtual Reality*, vol. 26, no. 1, pp. 77–89, Mar. 2022.
- [34] F. Scholkmann, S. Kleiser, A. J. Metz, R. Zimmermann, J. Mata Pavia, U. Wolf, and M. Wolf, "A review on continuous wave functional near-infrared spectroscopy and imaging instrumentation and methodology," *NeuroImage*, vol. 85 Pt 1, pp. 6–27, Jan. 2014.
- [35] "The present and future use of functional near-infrared spectroscopy (fNIRS) for cognitive neuroscience - Pinti - 2020 - Annals of the New York Academy of Sciences - Wiley Online Library," <https://nyaspubs.onlinelibrary.wiley.com/doi/full/10.1111/nyas.13948>.
- [36] S. S. Yeo, S. Y. Park, and S. H. Yun, "Investigating cortical activity during cybersickness by fNIRS," *Scientific Reports*, vol. 14, no. 1, p. 8093, Apr. 2024.
- [37] H. Yamamura, H. Baldauf, and K. Kunze, "HemodynamicVR - Adapting the User's Field Of View during Virtual Reality Locomotion Tasks to Reduce Cybersickness using Wearable Functional Near-Infrared Spectroscopy," in *Proceedings of the Augmented Humans International Conference 2021*, ser. AHs '21. New York, NY, USA: Association for Computing Machinery, Jul. 2021, pp. 223–227.
- [38] —, "Pleasant Locomotion – Towards Reducing Cybersickness using fNIRS during Walking Events in VR," in *Adjunct Publication of the 33rd Annual ACM Symposium on User Interface Software and Technology*. Virtual Event USA: ACM, Oct. 2020, pp. 56–58.
- [39] T. Tanimura, H. Takada, A. Sugiura, F. Kinoshita, and M. Takada, "Effects of The Low-Resolution 3D Video Clip on Cerebral Blood Flow Dynamics," *Advances in Science, Technology and Engineering Systems Journal*, vol. 4, no. 2, pp. 380–386, 2019.
- [40] A. Benelli, F. Neri, A. Cinti, P. Pasqualetti, S. M. Romanella, A. Giannotta, D. De Monte, M. Mandala, C. Smeralda, D. Prattichizzo, E. Santarnecchi, and S. Rossi, "Frequency-Dependent Reduction of Cybersickness in Virtual Reality by Transcranial Oscillatory Stimulation of the Vestibular Cortex," *Neurotherapeutics*, vol. 20, no. 6, pp. 1796–1807, Oct. 2023.
- [41] A. Abdalrhadi, K. Nitin, R. Lou, M. Moosavi, F. Merienne, M. Z. Yusoff, and N. Saad, "DEVELOPMENT OF VIRTUAL ENVIRONMENT TO ENHANCE USER EXPERIENCE WITH THE HELP OF ELECTROENCEPHALOGRAPHY,"
- [42] M.-H. Ahn, J. H. Park, H. Jeon, H.-J. Lee, H.-J. Kim, and S. K. Hong, "Temporal Dynamics of Visually Induced Motion Perception and Neural Evidence of Alterations in the Motion Perception Process in an Immersive Virtual Reality Environment," *Frontiers in Neuroscience*, vol. 14, p. 600839, Nov. 2020.
- [43] P. Andrievskaia, S. Berti, J. Spaniol, and B. Keshavarz, "Exploring neurophysiological correlates of visually induced motion sickness using electroencephalography (EEG)," *Experimental Brain Research*, vol. 241, no. 10, pp. 2463–2473, Oct. 2023.
- [44] J. K. Argasiński, N. Lipp, and S. Mazurek, "Electroencephalographic (EEG) Correlates of Visually Induced Motion Sickness (VIMS) in the Virtual Reality (VR) Based Simulations," in *Human-Computer Interaction – INTERACT 2023*, J. Abdelnour Nocera, M. Kristín Lárusdóttir, H. Petrie, A. Piccinno, and M. Winckler, Eds. Cham: Springer Nature Switzerland, 2023, vol. 14145, pp. 59–67.
- [45] C. Balaji, S. Mishra, and S. K. Satapathy, "Neurodynamic Mapping for Cybersickness Prediction in 3D Game Immersion," in *2025 International Conference on Emerging Systems and Intelligent Computing (ESIC)*. Bhubaneswar, India: IEEE, Feb. 2025, pp. 1–6.
- [46] L. M. Berger, G. Wood, C. Neuper, and S. E. Kober, "Sex Differences in User Experience in a VR EEG Neurofeedback Paradigm," in *Games and Learning Alliance*, F. De Rosa, I. Marfisi Schottman, J. Baalsrud Hauge, F. Bellotti, P. Dondio, and M. Romero, Eds. Cham: Springer International Publishing, 2021, vol. 13134, pp. 111–120.
- [47] L. M. Berger, G. Wood, and S. E. Kober, "Influence of a placebo tDCS treatment on cybersickness and EEG-neurofeedback success," *Behavioural Brain Research*, vol. 465, p. 114917, May 2024.
- [48] —, "Manipulating cybersickness in virtual reality-based neurofeedback and its effects on training performance," *Journal of Neural Engineering*, vol. 22, no. 2, p. 026014, Apr. 2025.
- [49] L. Chai, C. Hua, Z. Zhou, X. Chen, and J. Tao, "An EEG Study of Virtual Reality Motion Sickness Based on MVMD Combined with Entropy Asymmetry," in *Proceedings of 2023 Chinese Intelligent Automation Conference*, Z. Deng, Ed. Singapore: Springer Nature Singapore, 2023, vol. 1082, pp. 372–377.
- [50] E. Chang, I. Hwang, H. Jeon, Y. Chun, H. T. Kim, and C. Park, "Effects of rest frames on cybersickness and oscillatory brain activity," in *2013 International Winter Workshop on Brain-Computer Interface (BCI)*. Gangwon Province, Korea (South): IEEE, Feb. 2013, pp. 62–64.
- [51] E. Chang, H. T. Kim, and B. Yoo, "Identifying physiological correlates of cybersickness using heartbeat-evoked potential analysis," *Virtual Reality*, vol. 26, no. 3, pp. 1193–1205, Sep. 2022.
- [52] Y.-C. Chen, J.-R. Duann, C.-L. Lin, S.-W. Chuang, T.-P. Jung, and C.-T. Lin, "Motion-Sickness Related Brain Areas and EEG Power Activates," in *Foundations of Augmented Cognition. Neuroergonomics and Operational Neuroscience*, D. D. Schmorrow, I. V. Estabrooke, and M. Grootjen, Eds. Berlin, Heidelberg: Springer Berlin Heidelberg, 2009, vol. 5638, pp. 348–354.
- [53] Y.-C. Chen, J.-R. Duann, S.-W. Chuang, C.-L. Lin, L.-W. Ko, T.-P. Jung, and C.-T. Lin, "Spatial and temporal EEG dynamics of motion sickness," *NeuroImage*, vol. 49, no. 3, pp. 2862–2870, Feb. 2010.
- [54] F. Chiossi, Y. Weiss, T. Steinbrecher, C. Mai, and T. Kosch, "Mind the Visual Discomfort: Assessing Event-Related Potentials as Indicators for Visual Strain in Head-Mounted Displays," in *2024 IEEE International Symposium on Mixed and Augmented Reality (ISMAR)*. Bellevue, WA, USA: IEEE, Oct. 2024, pp. 150–159.
- [55] C. A. T. Cortes, C.-T. Lin, T.-T. N. Do, and H.-T. Chen, "An EEG-based Experiment on VR Sickness and Postural Instability While Walking in Virtual Environments," in *2023 IEEE Conference Virtual Reality and 3D User Interfaces (VR)*. Shanghai, China: IEEE, Mar. 2023, pp. 94–104.
- [56] Y. Daşdemir, "Classification of Emotional and Immersive Outcomes in the Context of Virtual Reality Scene Interactions," *Diagnostics*, vol. 13, no. 22, p. 3437, Nov. 2023.
- [57] —, "Locomotion techniques with EEG signals in a virtual reality environment," *Displays*, vol. 80, p. 102538, Dec. 2023.
- [58] Ö. S. Emekşiz, E. Erzincan, Y. Yemez, and T. M. Sezgin, "A Convolutional Transformer Model for EEG-Driven Cybersickness Detection in VR Experience," in *2025 33rd Signal Processing and Communications Applications Conference (SIU)*. Sile, Istanbul, Türkiye: IEEE, Jun. 2025, pp. 1–4.
- [59] N. Feng, B. Zhou, Q. Zhang, C. Hua, and Y. Yuan, "A comprehensive exploration of motion sickness process analysis from EEG signal and virtual reality," *Computer Methods and Programs in Biomedicine*, vol. 264, p. 108714, Jun. 2025.
- [60] M. Gallagher, R. Dowsett, and E. R. Ferrè, "Vection in virtual reality modulates vestibular-evoked myogenic potentials," *European Journal of Neuroscience*, vol. 50, no. 10, pp. 3557–3565, Nov. 2019.
- [61] C. Groth, J.-P. Tauscher, N. Heesen, M. Hattenbach, S. Castillo, and M. Magnor, "Omnidirectional Galvanic Vestibular Stimulation in Virtual Reality," *IEEE Transactions on Visualization and Computer Graphics*, vol. 28, no. 5, pp. 2234–2244, May 2022.
- [62] C. Hua, L. Chai, Y. Yan, J. Liu, Q. Wang, R. Fu, and Z. Zhou, "Assessment of Virtual Reality Motion Sickness Severity Based on EEG via LSTM/BiLSTM," *IEEE Sensors Journal*, vol. 23, no. 20, pp. 24 839–24 848, Oct. 2023.
- [63] C. Hua, J. Tao, Z. Zhou, L. Chai, Y. Yan, J. Liu, and R. Fu, "EEG classification model for virtual reality motion sickness based on multi-scale CNN feature correlation," *Computer Methods and Programs in Biomedicine*, vol. 251, p. 108218, Jun. 2024.
- [64] C. Hua, L. Chai, Z. Zhou, J. Tao, Y. Yan, X. Chen, J. Liu, and R. Fu, "Detection of virtual reality motion sickness based on EEG using asymmetry of entropy and cross-frequency coupling," *Physiology & Behavior*, vol. 284, p. 114626, Oct. 2024.
- [65] C. Hua, Z. Zhou, J. Tao, Y. Yan, J. Liu, R. Fu, and L. Chai, "A Siam-LSTM Model for Multichannel EEG on VR Motion Sickness

- Recognition," *IEEE Sensors Journal*, vol. 24, no. 21, pp. 35 245–35 254, Nov. 2024.
- [66] C. Hua, Y. Chen, J. Tao, Z. Dai, W. Yang, D. Chen, J. Liu, and R. Fu, "Dual-pathway EEG model with channel attention for virtual reality motion sickness detection," *Journal of Neuroscience Methods*, vol. 418, p. 110425, Jun. 2025.
- [67] K.-M. Jang, M. Kwon, S. G. Nam, D. Kim, and H. K. Lim, "Estimating objective (EEG) and subjective (SSQ) cybersickness in people with susceptibility to motion sickness," *Applied Ergonomics*, vol. 102, p. 103731, Jul. 2022.
- [68] D. Jeong, S. Yoo, and J. Yun, "Cybersickness Analysis with EEG Using Deep Learning Algorithms," in *2019 IEEE Conference on Virtual Reality and 3D User Interfaces (VR)*. Osaka, Japan: IEEE, Mar. 2019, pp. 827–835.
- [69] Khaitami, A. D. Wibawa, S. Mardi, S. Nugroho, and A. Z. Khoirunnisa, "EEG Visualization for Cybersickness Detection During Playing 3D Video Games," in *2019 International Seminar on Intelligent Technology and Its Applications (ISITIA)*. Surabaya, Indonesia: IEEE, Aug. 2019, pp. 325–330.
- [70] Y. Kim, H. Kim, H. Ko, and H. Kim, "Psychophysiological changes by navigation in a virtual reality," in *2001 Conference Proceedings of the 23rd Annual International Conference of the IEEE Engineering in Medicine and Biology Society*, vol. 4, Oct. 2001, pp. 3773–3776 vol.4.
- [71] Y. Y. Kim, H. J. Kim, E. N. Kim, H. D. Ko, and H. T. Kim, "Characteristic changes in the physiological components of cybersickness," *Psychophysiology*, vol. 42, no. 5, pp. 616–625, 2005.
- [72] J. Kim, W. Kim, H. Oh, S. Lee, and S. Lee, "A Deep Cybersickness Predictor Based on Brain Signal Analysis for Virtual Reality Contents," in *2019 IEEE/CVF International Conference on Computer Vision (ICCV)*. Seoul, Korea (South): IEEE, Oct. 2019, pp. 10 579–10 588.
- [73] J.-Y. Kim, J.-B. Son, H.-S. Leem, and S.-H. Lee, "Psychophysiological Alteration After Virtual Reality Experiences Using Smartphone-Assisted Head Mount Displays: An EEG-Based Source Localization Study," *Applied Sciences*, vol. 9, no. 12, p. 2501, Jun. 2019.
- [74] L.-W. Ko, C.-S. Wei, T.-P. Jung, and C.-T. Lin, "Estimating the Level of Motion Sickness Based on EEG Spectra," in *Foundations of Adaptive Systems. Directing the Future of Adaptive Systems*, D. D. Schmorow and C. M. Fidopiastis, Eds. Berlin, Heidelberg: Springer Berlin Heidelberg, 2011, vol. 6780, pp. 169–176.
- [75] L.-W. Ko, H.-C. Lee, S.-F. Tsai, T.-C. Shih, Y.-T. Chuang, H.-L. Huang, S.-Y. Ho, and C.-T. Lin, "EEG-based motion sickness classification system with genetic feature selection," in *2013 IEEE Symposium on Computational Intelligence, Cognitive Algorithms, Mind, and Brain (CCMB)*. Singapore, Singapore: IEEE, Apr. 2013, pp. 158–164.
- [76] S. Lee, S. Kim, H. G. Kim, and Y. M. Ro, "Assessing Individual VR Sickness Through Deep Feature Fusion of VR Video and Physiological Response," *IEEE Transactions on Circuits and Systems for Video Technology*, vol. 32, no. 5, pp. 2895–2907, May 2022.
- [77] Y. Li, A. Liu, and L. Ding, "Machine learning assessment of visually induced motion sickness levels based on multiple biosignals," *Biomedical Signal Processing and Control*, vol. 49, pp. 202–211, Mar. 2019.
- [78] X. Li, C. Zhu, C. Xu, J. Zhu, Y. Li, and S. Wu, "VR motion sickness recognition by using EEG rhythm energy ratio based on wavelet packet transform," *Computer Methods and Programs in Biomedicine*, vol. 188, p. 105266, May 2020.
- [79] G. Li, F. M. Varela, A. Habib, Q. Zhang, M. McGill, S. Brewster, and F. Pollick, "Exploring the feasibility of mitigating VR-HMD-induced cybersickness using cathodal transcranial direct current stimulation," in *2020 IEEE International Conference on Artificial Intelligence and Virtual Reality (AIVR)*. Utrecht, Netherlands: IEEE, Dec. 2020, pp. 123–129.
- [80] G. Li, O. Onuoha, M. McGill, S. Brewster, C. P. Chen, and F. Pollick, "Comparing Autonomic Physiological and Electroencephalography Features for VR Sickness Detection Using Predictive Models," in *2021 IEEE Symposium Series on Computational Intelligence (SSCI)*. Orlando, FL, USA: IEEE, Dec. 2021, pp. 01–08.
- [81] G. Li, K. Pohlmann, M. McGill, C. P. Chen, S. Brewster, and F. Pollick, "Exploring Neural Biomarkers in Young Adults Resistant to VR Motion Sickness: A Pilot Study of EEG," in *2023 IEEE Conference Virtual Reality and 3D User Interfaces (VR)*. Shanghai, China: IEEE, Mar. 2023, pp. 328–335.
- [82] G. Li, Y.-K. Wang, M. McGill, K. Pöhlmann, S. Brewster, and F. Pollick, "Resting-state EEG in the Vestibular Region Can Predict Motion Sickness Induced by a Motion-Simulated in-car VR Platform," in *2023 IEEE Symposium Series on Computational Intelligence (SSCI)*, Dec. 2023, pp. 47–52.
- [83] G. Li and T. Zanto, "Reduced VR motion sickness by applying random-phase transcranial alternating current stimulation to the left parietal cortex," *Brain Stimulation*, vol. 17, no. 3, pp. 550–552, May 2024.
- [84] C.-Y. Liao, S.-K. Tai, R.-C. Chen, and H. Hendry, "Using EEG and Deep Learning to Predict Motion Sickness Under Wearing a Virtual Reality Device," *IEEE Access*, vol. 8, pp. 126 784–126 796, 2020.
- [85] H. K. Lim, K. Ji, Y. S. Woo, D.-u. Han, D.-H. Lee, S. G. Nam, and K.-M. Jang, "Test-retest reliability of the virtual reality sickness evaluation using electroencephalography (EEG)," *Neuroscience Letters*, vol. 743, p. 135589, Jan. 2021.
- [86] C.-T. Lin, S.-W. Chuang, Y.-C. Chen, L.-W. Ko, S.-F. Liang, and T.-P. Jung, "EEG Effects of Motion Sickness Induced in a Dynamic Virtual Reality Environment," in *2007 29th Annual International Conference of the IEEE Engineering in Medicine and Biology Society*, Aug. 2007, pp. 3872–3875.
- [87] C.-T. Lin, S.-F. Tsai, H.-C. Lee, H.-L. Huang, S.-Y. Ho, and L.-W. Ko, "Motion sickness estimation system," in *The 2012 International Joint Conference on Neural Networks (IJCNN)*. Brisbane, Australia: IEEE, Jun. 2012, pp. 1–6.
- [88] Y.-T. Lin, Y.-Y. Chien, H.-H. Wang, F.-C. Lin, and Y.-P. Huang, "65-3: The Quantization of Cybersickness Level Using EEG and ECG for Virtual Reality Head-Mounted Display," *SID Symposium Digest of Technical Papers*, vol. 49, no. 1, pp. 862–865, May 2018.
- [89] R. Liu, E. Peli, and A. D. Hwang, "Measuring visually induced motion sickness using wearable devices," *Electronic Imaging*, vol. 29, no. 14, pp. 218–223, Jan. 2017.
- [90] R. Liu, M. Xu, Y. Zhang, E. Peli, and A. D. Hwang, "A Pilot Study on EEG-Based Evaluation of Visually Induced Motion Sickness," *The Journal of imaging science and technology*, vol. 64, no. 2, pp. 20 501–1–20 501–10, Mar. 2020.
- [91] M. Liu, B. Yang, M. Xu, P. Zan, L. Chen, and X. Xia, "Exploring quantitative assessment of cybersickness in virtual reality using EEG signals and a CNN-ECA-LSTM network," *Displays*, vol. 81, p. 102602, Jan. 2024.
- [92] M. Liu, B. Yang, P. Zan, L. Chen, B. Wang, and X. Xia, "Exploring the brain physiological activity and quantified assessment of VR cybersickness using EEG signals," *Displays*, vol. 85, p. 102879, Dec. 2024.
- [93] S. Luo, P. Ren, J. Wu, X. Wu, and X. Zhang, "Feature extraction method of EEG based on wavelet packet reconstruction and deep learning model of VR motion sickness feature classification and prediction," *PLOS ONE*, vol. 19, no. 7, p. e0305733, Jul. 2024.
- [94] M. Mawali, A. Z. Khoirunnisa, M. H. Purnomo, and A. D. Wibawa, "Classification of EEG Signal for Detecting Cybersickness through Time Domain Feature Extraction using Naïve Bayes," in *2018 International Conference on Computer Engineering, Network and Intelligent Multimedia (CENIM)*, Nov. 2018, pp. 29–34.
- [95] M.-A. Moïnnereau and T. H. Falk, "Cybersickness Marker Prediction Using a Biosensors-Instrumented VR Headset: A Pilot Study," in *2024 IEEE 4th International Conference on Human-Machine Systems (ICHMS)*. Toronto, ON, Canada: IEEE, May 2024, pp. 1–6.
- [96] Y. Nakata, M. Otsuki, M. Sakai, M. Nagai, R. Yamamoto, and A. Aoyama, "Exploring Display Parameters Associated with Cybersickness Using Electroencephalography," in *2025 IEEE Conference on Virtual Reality and 3D User Interfaces Abstracts and Workshops (VRW)*. Saint Malo, France: IEEE, Mar. 2025, pp. 1390–1391.
- [97] S. Nam, K.-M. Jang, M. Kwon, H. K. Lim, and J. Jeong, "Electroencephalogram microstates and functional connectivity of cybersickness," *Frontiers in Human Neuroscience*, vol. 16, p. 857768, Aug. 2022.
- [98] W. Nunes Da Silva, T. M. Porcino, C. D. Castanho, and R. P. Jacobi, "Analysis of Cybersickness through Biosignals: An approach with Symbolic Machine Learning," in *Symposium on Virtual and Augmented Reality*. Manaus Brazil: ACM, Sep. 2024, pp. 11–20.
- [99] A. Ozkan, U. Uyan, and U. Celikan, "Effects of speed, complexity and stereoscopic VR cues on cybersickness examined via EEG and self-reported measures," *Displays*, vol. 78, p. 102415, Jul. 2023.
- [100] E. S. Pane, A. Zuhriya Khoirunnisa, A. D. Wibawa, and M. H. Purnomo, "Identifying Severity Level of Cybersickness from EEG signals using CN2 Rule Induction Algorithm," in *2018 International Conference on Intelligent Informatics and Biomedical Sciences (ICI-IBMS)*, vol. 3, Oct. 2018, pp. 170–176.
- [101] G. N. Pradhan, R. C. Galvan-Garza, A. M. Perez, J. Stepanek, and M. J. Cevette, "Visual Vestibular Conflict Mitigation in Virtual Reality Using Galvanic Vestibular Stimulation," *Aerospace Medicine and Human Performance*, vol. 93, no. 5, pp. 406–414, May 2022.
- [102] R.-R. Qi, S.-F. Xiao, L.-L. Pan, Y.-Q. Mao, Y. Su, L.-J. Wang, and Y.-L. Cai, "Profiling of cybersickness and balance disturbance induced

- by virtual ship motion immersion combined with galvanic vestibular stimulation,” *Applied Ergonomics*, vol. 92, p. 103312, Apr. 2021.
- [103] M. Recenti, C. Ricciardi, R. Aubonnet, I. Picone, D. Jacob, H. Á. R. Svansson, S. Agnarsdóttir, G. H. Karlsson, V. Baeringsdóttir, H. Petersen, and P. Gargiulo, “Toward Predicting Motion Sickness Using Virtual Reality and a Moving Platform Assessing Brain, Muscles, and Heart Signals,” *Frontiers in Bioengineering and Biotechnology*, vol. 9, Apr. 2021.
- [104] O. Rosanne, D. Benesch, G. Kratzig, S. Paré, N. Bolt, and T. H. Falk, “To pre-process or not to pre-process? On the role of EEG enhancement for cybersickness characterization and the importance of amplitude modulation features,” *Frontiers in Virtual Reality*, vol. 6, p. 1468971, Jan. 2025.
- [105] J. Sameri, H. Coenegracht, S. Van Damme, F. De Turck, and M. Torres Vega, “Physiology-driven cybersickness detection in virtual reality: A machine learning and explainable AI approach,” *Virtual Reality*, vol. 28, no. 4, p. 174, Nov. 2024.
- [106] Z. Shen, X. Liu, W. Li, X. Li, and Q. Wang, “Classification of Visually Induced Motion Sickness Based on Phase-Locked Value Functional Connectivity Matrix and CNN-LSTM,” *Sensors*, vol. 24, no. 12, p. 3936, Jun. 2024.
- [107] M. Sra, A. Jain, and P. Maes, “Adding Proprioceptive Feedback to Virtual Reality Experiences Using Galvanic Vestibular Stimulation,” in *Proceedings of the 2019 CHI Conference on Human Factors in Computing Systems*. Glasgow Scotland UK: ACM, May 2019, pp. 1–14.
- [108] G. R. F. Suwandi, S. N. Khotimah, F. Haryanto, and Suprijadi, “Electroencephalography Signal Analysis for Virtual Reality Sickness: Head-mounted Display and Screen-based,” *International Journal on Advanced Science, Engineering and Information Technology*, vol. 13, no. 4, pp. 1449–1455, Aug. 2023.
- [109] G. R. F. Suwandi, S. N. Khotimah, F. Haryanto, and Suprijadi, “Exploring the Impact of Motion Parameter Variations in Virtual Reality Content on Visually Induced Motion Sickness: An Electroencephalography Signal Analysis Approach,” *International Journal of Online and Biomedical Engineering (iJOE)*, vol. 20, no. 12, pp. 160–176, Sep. 2024.
- [110] N. Takeuchi, T. Mori, Y. Suzukamo, and S.-I. Izumi, “Modulation of Excitability in the Temporoparietal Junction Relieves Virtual Reality Sickness,” *Cyberpsychology, Behavior, and Social Networking*, vol. 21, no. 6, pp. 381–387, Jun. 2018.
- [111] N. Tian and R. Boulic, “Who says you are so sick? An investigation on individual susceptibility to cybersickness triggers using EEG, EGG and ECG,” *IEEE Transactions on Visualization and Computer Graphics*, pp. 1–11, 2024.
- [112] U. Uyan and U. Celikcan, “CDMS: A real-time system for EEG-guided cybersickness mitigation through adaptive adjustment of VR content factors,” *Displays*, vol. 83, p. 102704, Jul. 2024.
- [113] K. Watanabe, K. Onishi, M. Koeda, M. Tsujishita, and H. Noborio, “Study of VR Sickness Reduction Effect by Tunneling Effect According to the Gazing Point,” in *Human-Computer Interaction*, M. Kurosu and A. Hashizume, Eds. Cham: Springer Nature Switzerland, 2025, vol. 15767, pp. 446–460.
- [114] S. Weech, J. Moon, and N. F. Troje, “Influence of bone-conducted vibration on simulator sickness in virtual reality,” *PLOS ONE*, vol. 13, no. 3, p. e0194137, Mar. 2018.
- [115] S. Weech, T. Wall, and M. Barnett-Cowan, “Reduction of cybersickness during and immediately following noisy galvanic vestibular stimulation,” *Experimental Brain Research*, vol. 238, no. 2, pp. 427–437, Feb. 2020.
- [116] C.-S. Wei, L.-W. Ko, S.-W. Chuang, T.-P. Jung, and C.-T. Lin, “EEG-based evaluation system for motion sickness estimation,” in *2011 5th International IEEE/EMBS Conference on Neural Engineering*. Cancun: IEEE, Apr. 2011, pp. 100–103.
- [117] Y. S. Woo, K.-M. Jang, S. G. Nam, M. Kwon, and H. K. Lim, “Recovery time from VR sickness due to susceptibility: Objective and quantitative evaluation using electroencephalography,” *Heliyon*, vol. 9, no. 4, p. e14792, Apr. 2023.
- [118] J. Wu, Q. Zhou, J. Li, X. Kong, and Y. Xiao, “Inhibition-related N2 and P3: Indicators of visually induced motion sickness (VIMS),” *International Journal of Industrial Ergonomics*, vol. 78, p. 102981, Jul. 2020.
- [119] W. Xu, H.-N. Liang, Y. Yu, D. Monteiro, K. Hasan, and C. Fleming, “Assessing the effects of a full-body motion-based exergame in virtual reality,” in *Proceedings of the Seventh International Symposium of Chinese CHI*. Xiamen China: ACM, Jun. 2019, pp. 1–6.
- [120] M. Xu, B. Yang, M. Liu, Y. Huang, X. Xia, and J. Zhang, “SE-CNN Attention Structure for Quantitative EEG-Based Assessment of VR Motion Sickness,” in *Proceedings of the 2023 9th International Conference on Computing and Artificial Intelligence*, ser. ICCAI ’23. New York, NY, USA: Association for Computing Machinery, Aug. 2023, pp. 606–611.
- [121] A. H. X. Yang, N. K. Kasabov, and Y. O. Cakmak, “Prediction and detection of virtual reality induced cybersickness: A spiking neural network approach using spatiotemporal EEG brain data and heart rate variability,” *Brain Informatics*, vol. 10, no. 1, p. 15, Dec. 2023.
- [122] S. S. Yeo, J. W. Kwon, and S. Y. Park, “EEG-based analysis of various sensory stimulation effects to reduce visually induced motion sickness in virtual reality,” *Scientific Reports*, vol. 12, no. 1, p. 18043, Oct. 2022.
- [123] Z. Zhou, C. Hua, L. Chai, and J. Tao, “Application of EEG S-Transformation Combined with Dimensionless Metrics for Automatic Detection of Cybersickness,” in *Proceedings of 2023 Chinese Intelligent Automation Conference*, Z. Deng, Ed. Singapore: Springer Nature Singapore, 2023, vol. 1082, pp. 634–640.
- [124] A. Z. Khoirunnisa, E. S. Pane, A. D. Wibawa, and M. H. Purnomo, “Channel Selection of EEG-Based Cybersickness Recognition during Playing Video Game Using Correlation Feature Selection (CFS),” in *2018 2nd International Conference on Biomedical Engineering (IBIOMED)*, Jul. 2018, pp. 48–53.
- [125] Z. Li, L. Zhao, J. Chang, W. Li, M. Yang, C. Li, R. Wang, and L. Ji, “EEG-based evaluation of motion sickness and reducing sensory conflict in a simulated autonomous driving environment,” in *2022 44th Annual International Conference of the IEEE Engineering in Medicine & Biology Society (EMBC)*, Jul. 2022, pp. 4026–4030.
- [126] E. Molefi, R. Palaniappan, and I. McLoughlin, “Vibro-motor Reprocessing Therapy towards Managing Motion Sickness Reduction: Evidence from EEG,” *Annual International Conference of the IEEE Engineering in Medicine and Biology Society. IEEE Engineering in Medicine and Biology Society. Annual International Conference*, vol. 2022, pp. 4781–4784, Jul. 2022.
- [127] Y. Wei, Y. O. Okazaki, R. H. So, W. C. Chu, and K. Kitajo, “Motion sickness-susceptible participants exposed to coherent rotating dot patterns show excessive N2 amplitudes and impaired theta-band phase synchronization,” *NeuroImage*, vol. 202, p. 116028, Nov. 2019.
- [128] B. Ren and Q. Zhou, “Assessing Passengers’ Motion Sickness Levels Based on Cerebral Blood Oxygen Signals and Simulation of Actual Ride Sensation,” *Diagnostics*, vol. 13, no. 8, p. 1403, Apr. 2023.
- [129] Y.-H. Yu, P.-C. Lai, L.-W. Ko, C.-H. Chuang, B.-C. Kuo, and C.-T. Lin, “An EEG-based classification system of Passenger’s motion sickness level by using feature extraction/selection technologies,” in *The 2010 International Joint Conference on Neural Networks (IJCNN)*, Jul. 2010, pp. 1–6.
- [130] C.-S. Wei, L.-W. Ko, S.-W. Chuang, T.-P. Jung, and C.-T. Lin, “Genetic feature selection in EEG-based motion sickness estimation,” in *The 2011 International Joint Conference on Neural Networks*, Jul. 2011, pp. 365–369.
- [131] C.-S. Wei, S.-W. Chuang, W.-R. Wang, L.-W. Ko, T.-P. Jung, and C.-T. Lin, “Implementation of a motion sickness evaluation system based on EEG spectrum analysis,” in *2011 IEEE International Symposium of Circuits and Systems (ISCAS)*, May 2011, pp. 1081–1084.
- [132] B. U. Demirel, A. H. Dogan, J. Rossie, M. Möbus, and C. Holz, “Beyond Subjectivity: Continuous Cybersickness Detection Using EEG-based Multitaper Spectrum Estimation,” *IEEE Transactions on Visualization and Computer Graphics*, vol. 31, no. 5, pp. 2525–2534, May 2025.
- [133] L. Zhou, H. Hu, B. Qin, Q. Zhu, and Z. Qian, “Brain activity differences between susceptible and non-susceptible populations under visually induced motion sickness based on sensor-space and source-space analyses,” *Brain Research*, vol. 1815, p. 148474, Sep. 2023.
- [134] J.-R. Park, D.-W. Lim, S.-Y. Lee, H.-W. Lee, M.-H. Choi, and S.-C. Chung, “Long-Term Study of Simulator Sickness: Differences in EEG Response Due to Individual Sensitivity,” *International Journal of Neuroscience*, vol. 118, no. 6, pp. 857–865, Jun. 2008.
- [135] J. W. Kelly, S. B. Gilbert, M. C. Dorneich, and K. A. Costabile, “Gender differences in cybersickness: Clarifying confusion and identifying paths forward,” in *2023 IEEE Conference on Virtual Reality and 3D User Interfaces Abstracts and Workshops (VRW)*. Shanghai, China: IEEE, Mar. 2023, pp. 283–288.
- [136] B. Lawson and J. Bolkhovsky, “Attention Military/Commercial Simulation Developers, Users, & Trainers: Visually-induced or Motion-induced Sickness is not Necessarily More Severe for Women,” in *14th*

International Conference on Applied Human Factors and Ergonomics (AHFE 2023), 2023.

- [137] P. Kourtesis, R. Amir, J. Linnell, F. Argelaguet, and S. E. MacPherson, "Cybersickness, Cognition, & Motor Skills: The Effects of Music, Gender, and Gaming Experience," *IEEE Transactions on Visualization and Computer Graphics*, vol. 29, no. 5, pp. 2326–2336, May 2023.
- [138] E. Molefi, I. McLoughlin, and R. Palaniappan, "Transcutaneous Auricular Vagus Nerve Stimulation for Visually Induced Motion Sickness: An eLORETA Study," *Brain Topography*, vol. 38, no. 1, p. 11, Jan. 2025.
- [139] S. Palmisano, D. Apthorp, T. Seno, and P. J. Stapley, "Spontaneous postural sway predicts the strength of smooth vection," *Experimental Brain Research*, vol. 232, no. 4, pp. 1185–1191, Apr. 2014.
- [140] P. Kourtesis, J. Linnell, R. Amir, F. Argelaguet, and S. E. MacPherson, "Cybersickness in Virtual Reality Questionnaire (CSQ-VR): A Validation and Comparison against SSQ and VRSQ," *Virtual Worlds*, vol. 2, no. 1, pp. 16–35, Mar. 2023.
- [141] T. Saito and M. Rehmsmeier, "The Precision-Recall Plot Is More Informative than the ROC Plot When Evaluating Binary Classifiers on Imbalanced Datasets," *PLOS ONE*, vol. 10, no. 3, p. e0118432, Mar. 2015.
- [142] E. Combrisson and K. Jerbi, "Exceeding chance level by chance: The caveat of theoretical chance levels in brain signal classification and statistical assessment of decoding accuracy," *Journal of Neuroscience Methods*, vol. 250, pp. 126–136, Jul. 2015.
- [143] G. Varoquaux, "Cross-validation failure: Small sample sizes lead to large error bars," *NeuroImage*, vol. 180, no. Pt A, pp. 68–77, Oct. 2018.
- [144] H. Berger, "Über das Elektrenkephalogramm des Menschen," *Archiv für Psychiatrie und Nervenkrankheiten*, vol. 87, no. 1, pp. 527–570, Dec. 1929.
- [145] W. Klimesch, P. Sauseng, and S. Hanslmayr, "EEG alpha oscillations: The inhibition-timing hypothesis," *Brain Research Reviews*, vol. 53, no. 1, pp. 63–88, Jan. 2007.
- [146] R. J. Barry, A. R. Clarke, S. J. Johnstone, C. A. Magee, and J. A. Rushby, "EEG differences between eyes-closed and eyes-open resting conditions," *Clinical Neurophysiology*, vol. 118, no. 12, pp. 2765–2773, Dec. 2007.
- [147] C. Zhang, S. Li, Y. Li, S. E. Li, and B. Nie, "Analysis of Motion Sickness Associated Brain Activity Using fNIRS: A Driving Simulator Study," *IEEE Access*, vol. 8, pp. 207 415–207 425, 2020.
- [148] M.-A. Moynereau, A. A. Oliveira, and T. H. Falk, "Instrumenting a virtual reality headset for at-home gamer experience monitoring and behavioural assessment," *Frontiers in Virtual Reality*, vol. 3, Oct. 2022.